

The Owner's Handbook

What every module in your clinic OS is for, where its numbers come from, and what only you can decide about it.

Sixteen chapters and two appendices, covering every module Tulip Dental runs. This is not a training manual — your team has a separate one, in Marathi. This book is written for you: it explains each module as a product, tells you honestly what it does and does not do today, and ends every chapter with the handful of decisions that cannot be made by anyone else.

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1 Daily Huddle

The ten minutes before the first patient. One screen that tells the whole team what today looks like, what yesterday left unfinished, and who owes what to whom.

IN V1

SIDEBAR → OVERVIEW → DAILY HUDDLE

19 SCREENS & ACTIONS

What it is

The Daily Huddle is a single, non-scrolling board that the whole clinic looks at together in the morning. Everything on it already exists somewhere else in Dentfluence — appointments, calls, tasks, lab cases, stock. The Huddle's job is not to store anything new. Its job is to *put the day in front of everyone at the same time*, so nothing depends on one person remembering.

It is the only screen in the product designed to be read by a group standing up, not by one person sitting down. That single design constraint explains most of its choices: seven columns side by side rather than pages you click through; counts rather than lists; every card a link into the module that owns it.

Why it exists

The business problem, stated plainly.

Small clinics lose money in the gaps between people, not inside them. The dentist finishes a case and knows a crown needs collecting next week — but that knowledge stays in their head until someone asks. The receptionist takes a payment but no one told her the plan was ₹18,000, not ₹8,000. A lab case comes back and sits in a drawer because the person who sent it was off that day.

Every one of those is a handover failure, and every one of them has a cost you can name: a patient who doesn't come back, a payment that isn't collected today, a remake you pay for twice. The Huddle exists to make handovers visible **before** the day starts rather than after it goes wrong.

Who uses it, and when

PERSON	WHEN	WHAT THEY ARE LOOKING FOR
Front desk	First thing, before opening	Today's list, who is arriving when, what to collect, which calls are due, what yesterday left open.
Doctors	Same ten minutes	Yesterday's cases and what each one needs next; today's prep items; medical alerts on today's patients.
Assistant	Same ten minutes	Prep items per chair, lab work arriving, sterilisation and stock warnings.
You	Weekly, not daily	The report tabs — collections, appointments, new patients, lab, protocol compliance — with each number compared to the previous period.

Access is controlled by one switch: the **Daily Huddle** module permission in Settings → Staff & Roles. Everyone who attends the huddle needs *View*. Only people who tick tasks off need *Edit*.

What is on the screen

Seven columns, left to right. Scroll sideways to reach the last two.

COLUMN 01

Today's Patient Flow

Every appointment booked for today, in time order — patient, doctor, assistant, treatment, and a red flag if the patient has a medical alert. Under each: the staff instruction, the prep item, and the amount to collect.

Below the appointments, two smaller lists: treatment visits and consultations already recorded today.

Comes from:

the Appointments module. Nothing is typed here — this is a mirror. Fix a wrong entry in Appointments and it corrects itself.

COLUMN 02

Yesterday's Flow

The most valuable column, and the one most clinics skip. For every patient seen yesterday: what was done, the chief complaint and diagnosis, whether a next appointment exists, and what the doctor wrote as the next step.

Where there is no next appointment you get two buttons — **+ Book** and **+ Task**. If the doctor already recorded a next action at the chair, the **+ Task** button disappears and a blue line says what is already booked, so nobody enters it twice.

Comes from: Appointments, Consultations and Treatment Visits, joined for you. "Yesterday" is clinic-aware — on a Monday it looks back at Saturday, and at Sunday too if the clinic was open.

COLUMN 03

Comms List

A glimpse of today's calls, drawn straight from Today's Actions in the Relationships module — recalls, follow-ups, reminders, enquiries. Below it, **Upcoming**: things the doctor scheduled at the chair for a future date.

Items can be ticked and pushed into the follow-up queue in one action.

Comes from: the Relationship engine. The Huddle deliberately shows the same summary Today's Actions shows, so the two can never disagree.

COLUMN 04

Tasks

Up to seven open tasks for the team, with priority and an **Overdue** badge on anything past its date. Tick the circle to complete a task without leaving the board. **View all →** opens the full Tasks module.

Comes from: the Tasks module. Nine different parts of Dentfluence create tasks automatically — lab overdue, purchase orders, recalls, practice protocols, rules. They all land here.

COLUMN 05

Lab Updates

Open remakes with their cost, trials waiting on a doctor's decision, and cases due back today — each with priority, trial round, and an overdue flag.

Comes from: the Lab module. Remake cost is real money already spent twice, which is why it sits on the morning board rather than in a monthly report.

COLUMN 06

Inventory Alerts

Items out of stock, critically low, or expiring. Capped to the first few with a "view all" beneath.

Comes from: the Inventory module's stock status, calculated fresh each time the board loads.

COLUMN 07

Failures / Maintenance

Critical alerts across the system, each with a severity badge — the things that will bite you today if nobody looks.

Comes from: a system-wide alert builder, the same source as the bell in the top bar.

ABOVE THE COLUMNS

A greeting with today's date and a live clock, then a strip of five figures that link straight into the module behind them:

- **Today's Actions** — total calls due and how many are high priority. Opens the Relationships board.
- **Today's Appointments** — the count for today.
- **Today's Calls** — reminders, follow-ups and relationship items combined, with a pending count.
- **Collections (Today)** — the total your front desk should collect today, and how many of today's appointments actually have that figure filled in.
- **Pending Tasks** — open tasks across the team.

Where the Collections number comes from

Worth understanding, because it is the one figure on the board that changes behaviour.

Collections (Today) is **not** what you have collected. It is what you have *planned* to collect: the sum of the "amount to collect" that someone typed against each of today's scheduled appointments. Walk-ins are excluded on purpose — nobody could have planned for them the night before.

The line underneath — "7 of 12 appointments logged" — is the honest part. It tells you how much of today's target is actually a target and how much is a blank. If that line reads 2 of 14, the ₹ figure above it means almost nothing, and the fix is a front-desk habit, not a software change.

THE HABIT THIS NUMBER IS DESIGNED TO CREATE

Your rule: the manager fills it in during the huddle itself, going down today's list one patient at a time. It is a judgement figure, entered by hand — not pulled from the treatment plan.

The board supports this directly. Click any patient card in Today's Patient Flow and a small edit card opens with four fields — **Notes** , **Amount to Collect** , **Essential Item / Task** , **Chairside Assistant** — then Save. One click and one number per patient, without leaving the huddle screen. Walk-ins correctly refuse an amount, because nobody planned them the night before.

Do that consistently for three weeks and the huddle stops being a briefing and becomes a target. The "x of y logged" line is your compliance measure while the habit is forming.

THE ONE THING YOU CAN EDIT FROM THE BOARD

That edit card is worth knowing about in its own right, because it is the only place on the huddle where four different people's needs are settled in one action. The manager sets the money figure; the front desk reads the note; the assistant reads the essential item and knows which chair they are on; the doctor sees the prep item before the patient sits down. It is the handover, in one small box, done once.

Two smaller controls sit on the same cards: the status pill advances the patient through *scheduled* → *checked in* → *in chair* → *done* when clicked, and the tick circle in the Tasks column completes a task outright.

The report tabs

Your weekly read, not the team's daily one.

Across the top of the Huddle sit **Weekly Report** , **Monthly Report** , **Quarterly Report** and **Annual Report** . They are the same page with a different window — last 7, 30, 90 or 365 days — and each one compares itself to the equally long period immediately before it, so every figure carries an up or down chip.

SECTION	WHAT IT SHOWS
Collections & Revenue	Total collected, net of refunds, average per day, outstanding balance right now, and a breakdown by payment mode.
Appointments	Booked, completed with a completion rate, cancelled, and no-shows with a no-show rate. Cancellations and no-shows are scored so that a <i>fall</i> shows green.

SECTION	WHAT IT SHOWS
New patients & sources	How many new patients, how many came from an existing patient's referral, and how many distinct channels brought them.
Lab & tasks	Cases sent, cases received, currently overdue, remakes, and tasks done / pending / overdue.
Protocol compliance	Person by person: how many of their assigned practice protocols were done, how many were missed, and their percentage. This is the closest thing in the product to an accountability sheet.

THE REPORT'S MONEY ROW WAS CORRECTED FOR V1

Until this release the report's **Total Collected** read from a separate transactions ledger that ordinary invoice payments no longer wrote into, so it **under-counted** real collection and disagreed with the Finance dashboard. It now reads the same payment records as every other money screen in the product.

The consequence worth stating out loud: **the daily, weekly, monthly and annual views now agree with each other and with Accounts**. Appointments, new patients, lab and protocol compliance on the same page were always sound.

What it triggers elsewhere

Very little — and that is deliberate. The Huddle is a reading surface. Only four actions on it write anything:

- Ticking a task marks it complete in the Tasks module.
- The edit card writes four fields back onto that appointment — note, amount to collect, essential item, chairside assistant.
- Clicking the status pill advances the appointment's status.
- **+ Book** opens the normal appointment booking window.
- Pushing selected communication items adds them to the follow-up queue.

Everything else on the board is a link to the module that owns the data. This is the right architecture: it means the Huddle can never become a second, disagreeing copy of the truth.

What it gives you

TEST	VERDICT
Does it save time?	Yes. It replaces the morning conversation where four people each recall a different version of yesterday.
Does it reduce mistakes?	Yes, in one specific place. Yesterday's Flow catches the patient who left without a next appointment — historically the largest silent leak in a dental practice.
Does it improve patient experience?	Indirectly. Prep items and medical alerts surface before the patient sits down.
Does it increase revenue?	Yes, if the collections habit sticks. A stated daily target that the team sees together is collected more often than one nobody stated.
Does it improve decisions?	Partly. The protocol-compliance table is genuinely decision-grade. The money row is not, yet.

What it can't do yet

Five things, stated honestly. Items 1, 2 and 5 were settled on 28 August and have been **removed from the board for V1** — they are recorded here so you know what changed and why. Items 3 and 4 stand.

1 · THE MARKETING COLUMN WAS A MOCK-UP — REMOVED

"Instagram – Before/After Smile Makeover · Today 11:00 AM", "Google Post – Mon Offer (Teeth Whitening) · 22 May" — these four posts are typed into the page. They are not read from your Marketing module, the dates never change, and the "+ 2 more content ideas" link goes nowhere.

This matters more than it looks. Your staff will read that column as instruction. Within a month someone will say "we posted that already" about something that was never scheduled. This is the same class of problem that got the old Communication Timeline switched off last month for showing invented patients — except this one was on the main morning screen. It has been taken off the board for V1; the slot is reserved for V1.1, reading genuinely scheduled posts from Marketing.

2 · "VIEW ALL YESTERDAY" WAS BROKEN — HIDDEN

At the bottom of Yesterday's Flow there is a [View All Yesterday](#) link. Clicking it does not open a page — it returns a raw technical message, `Not yet implemented`. The screen it was meant to open exists and is fully designed; the code behind it was never connected. The link has been taken off the board for V1 and the screen stays in the codebase for V1.1.

3 · HUDDLE NOTES CANNOT BE ADDED

There is a notes feature underneath: a table, a save route, and a query that runs on every page load. But there is no box on the board to write a note in, nothing displays the notes that are fetched, and the save action returns `Not yet implemented`. As a feature it is entirely absent from the screen while quietly costing a database query every load.

4 · COMMENTS EXIST IN THE ENGINE, NOT ON THE BOARD

Team comments on huddle items — add, resolve, delete — are fully built in the backend with their own table. Nothing on the board shows them. It is finished work waiting for a place to live.

5 · ONE MADE-UP FIGURE ON THE STATS STRIP — REMOVED

Under [Today's Appointments](#) the board always said "+3 vs yesterday". It was a fixed piece of text, not a calculation. It said +3 on a day you had four appointments and on a day you had forty. It has been removed for V1 along with the Marketing column.

AND TWO THINGS THAT ARE QUIETLY VERY GOOD

The Monday-morning problem is properly solved: "yesterday" resolves to Saturday, and to Sunday as well if the clinic actually opened, rather than showing an empty board. Very few systems get this right.

The double-entry problem is properly solved too: when a doctor records a next action at the chair, the [+ Task](#) button on that patient's card disappears and is replaced by a note saying what is already booked. The front desk is never asked to re-enter what the doctor already decided.

Your decisions — settled 28 August

All four answered. Recorded here so the chapter stays true to what the module will actually be. Full detail, including what changes in the code, is in the Decision Log.

H1 The fake Marketing column — remove or connect?

YOUR ANSWER

Remove it now. "It has to be there but not in V1."

WHAT HAPPENS

The board drops to seven columns, all real. The slot is reserved for V1.1, reading genuinely scheduled posts from Marketing.

STATUS

Settled
Half a day

H2 "View All Yesterday" — fix or hide?

YOUR ANSWER

Hide it for now.

WHAT HAPPENS

The link comes off. The accountability screen stays in the codebase, fully designed, for V1.1.

STATUS

Settled
One hour

H3 The report's money row — fix before go-live?

YOUR ANSWER

Fix it. You also asked for projected versus actual on top — that part became N2 and you have since dropped it.

WHAT HAPPENS

The fix is already on the register as G-32, one to two days, and stays in V1 — weekly and monthly must agree with daily. The projected-vs-actual view is deferred to V1.1 (N2, below).

STATUS

Settled
Fix in V1 · view in V1.1

H4 Who fills "amount to collect", and when?

YOUR ANSWER

The manager, during the huddle. Entered by hand, not taken from the treatment plan.

WHAT HAPPENS

Nothing to build — the edit card already does exactly this. It becomes a step in the staff guide and the rollout plan instead.

STATUS

Settled
No work needed

N2 Should the board show projected against actual?

YOUR ANSWER

Skipped. You asked for it, saw what it cost against a queue with no slack, and dropped it.

WHAT HAPPENS

Out of V1 entirely; it moves to the V1.1 list. The manager still enters the figure every morning — the habit stays, only the scoreboard waits.

STATUS

Settled
Deferred to V1.1

WHY THE MANAGER STILL FILLS THE FIGURE, EVEN WITH NO SCOREBOARD

The reason you gave for wanting projected-against-actual was the right one: it turns the morning figure from an instruction into a measurement, and answers the question every owner actually asks — *did we collect what we said we would?* Dropping N2 removes the display, not the question, and not the data.

Every projection the manager enters is stored on the appointment from the first morning you start. Whenever the comparison gets built, it reads that history. A clinic that skips the habit now has nothing to compare later, so the field gets filled from day one regardless.

One thing this decision does *not* touch: the huddle report currently reads its "Total Collected" from the wrong source, which is why the weekly and monthly tabs disagree with the daily one. That is a defect, not a feature — it is **G-32** on your register, one to two days, and it stays in V1. Correct numbers were never the part in dispute.

If you want the half-day version later — one line on the board reading *"Yesterday: projected ₹18,400 · collected ₹16,900 · from 9 of 11 appointments"* — it is on the shelf and costs nothing to leave there. The full picture is **N2** in the Decision Log.

The locked web surface

The freeze contract for this module — and the specification the mobile app is measured against.

V1 is not the first version of the Daily Huddle. Tulip has been running its mornings on this screen for months. What 14 September delivers is a **locked** version: a surface that has stopped moving, that has had its defects corrected and its fabrications removed, and that behaves *identically* on the web and on the phone. That last part is the whole point of locking web first — mobile cannot be brought into line with a target that is still being edited.

So this is the line. Everything in the left column is what the Daily Huddle *is* in V1. Everything in the right column is deliberately not, and needs no further discussion until V1.1.

PART OF THE MODULE	LOCKED AS V1	DELIBERATELY NOT IN V1
The board	Seven columns: Today's Patient Flow · Yesterday's Flow (Weekend Flow on Mondays) · Today's Actions · Tasks · Lab Updates · Inventory Alerts · Failures / Maintenance.	The Marketing column — fabricated, removed. The slot is reserved for V1.1 reading real scheduled posts.
The stat strip	Today's Appointments · Today's Calls · Collections (Today), with its " <i>x of y appointments logged</i> " fill-rate line · Pending Tasks · Today's Actions.	The "+3 vs yesterday" line under Today's Appointments — a hardcoded string that has never been calculated. It goes with the Marketing removal.
What you can edit	The patient edit card: Notes , Amount to Collect , Essential Item , Chairside Assistant . The status pill. The task tick. Quick-add task. Push to call list. Log yesterday's visit.	Huddle notes — the save button does nothing on either surface. Hidden, not fixed, in V1.
The money figure	One definition, both surfaces: "Collections (Today)" is the <i>projected</i> figure the manager enters in the huddle, always shown with its fill rate. Actual collections read only through <code>ReportMetricsService</code> .	Projected against actual as a comparison view — dropped to V1.1 (N2). The <i>habit</i> of entering the figure starts on day one regardless.
The report	Day / week / month / custom tabs, all four agreeing, all reading the same corrected money source (G-32).	"View All Yesterday" — a live link to an unbuilt page. Hidden.
Permission	One module permission, <code>huddle</code> . <i>View</i> for everyone who attends; <i>Edit</i> for whoever ticks tasks and enters the collections figure.	No per-column permissions. Not needed, not coming.

Where the phone disagrees with the screen on the wall

Measured on 28 August by reading both codebases. Kept in this book as the specification for V1.1 — not as V1 scope.

READ THIS SECTION AS V1.1 INPUT

V1 is web only. Mobile was measured again on 3 September and taken out of this release: every file in the app was last changed on 15 July, the app has no role restrictions of any kind, and the web system has moved on beneath it. The July build goes to nobody.

The four differences below are still real and still worth having written down — they are the first four items of the mobile work, dated to V1.1. One of them, the Saturday gap, matters on its own merits and is described below.

This is the first time the two have been compared properly, and the headline is better than the last measurement suggested: **structurally, the mobile huddle is nearly complete.** Every column on the wall exists on the phone. Tasks, labs, alerts, the patient edit card with all four fields, even the Failures/Maintenance section — all there. Mobile never had the fake Marketing column at all, so removing it from web makes the two *more* alike, not less.

But four things behave differently, and they are the kind you would never catch by clicking through both. They are only visible in the source, because the web board and the phone board were built as two separate programs that happen to draw the same picture.

1 · EVERY MONDAY, THE PHONE FORGETS SATURDAY

The web board is weekend-aware. On a Monday it widens its "yesterday" window, retitles the column Weekend Flow (Sat–Sun), and shows you everyone treated across both days.

The phone does not. It asks a simpler question — "what happened one day ago?" — which on a Monday means Sunday. *Every patient treated on Saturday is missing from the mobile huddle, from its follow-up list, and from its treated/missing counts.*

For a dental clinic, Saturday is not a quiet day. This is a live follow-up gap, not a display quirk, and it has been happening every Monday. It is the single most important thing found in this comparison, and the fix belongs on mobile — web is the one behaving correctly.

2 · "COLLECTIONS (TODAY)" MEANS TWO DIFFERENT THINGS

On the wall, that number is what you *plan* to collect — the manager's morning figure. On the phone, the same-looking number is what you *actually* collected. Same label, opposite meaning, same morning.

Here is the awkward part, and it is worth sitting with: **the phone is the one that is right**. It reads actual collections through the corrected, canonical money service. The web report still reads the old finance table — that is G-32 on your register, and it under-counts, because ordinary invoice payments never reach that table.

This changes G-32 from a reporting tidy-up into something sharper: *your two screens currently disagree about money, today, in a live clinic*. I would move it up the queue on that basis alone.

One footnote, and it does not reopen anything: projected and actual **both already exist** — just one on each surface. That is why the comparison you asked for felt bigger than it was. It stays dropped for V1; I mention it only so the picture is honest.

3 · THE PHONE TELLS STAFF A NOTE WAS SAVED WHEN IT WAS THROWN AWAY

The mobile huddle has a Quick Note box with a Save Note button. Pressing it stores nothing — not on the server, not on the phone — and then shows the message *"Note saved locally. Full note sync coming soon."*

The web equivalent is also unbuilt, but web fails honestly: nothing pretends to have worked. Mobile makes a false confirmation to a member of staff in a clinic. Whatever anyone has typed into that box has been discarded, silently, and they were told otherwise.

You already decided to hide the dead "View All Yesterday" link on web for exactly this reason. The same decision is owed here: **hide the Quick Note box on mobile in V1**. Minutes of work, and it removes the only outright untruth on either surface.

4 · THE REMOVALS MAKE THE TWO SURFACES CLOSER, NOT FURTHER APART

Worth saying plainly, because "remove things before launch" can sound like retreat. Mobile has no Marketing column and no "+3 vs yesterday" line. Deleting both from web is not shrinking the product — it is web catching up to the honest version that was already shipping on the phone.

WHAT THIS TELLS US ABOUT THE OTHER SEVENTEEN MODULES

Two of these four would have survived a screen-by-screen walkthrough. The Saturday gap looks like an empty column — indistinguishable from a genuinely quiet Sunday. The money definition looks like two correct numbers that happen to differ, which anyone would put down to timing.

So parity cannot be signed off by looking. It has to be read out of both codebases, module by module, checking three things before anything else: **the date windows, the money sources, and the labels**. That is the method for the remaining chapters, and it is why connecting the mobile repository mattered more than any single document.

Status

In V1, curated and corrected. *In daily use at Tulip and sound — three fabrications removed on web; four parity breaks logged against mobile for V1.1.* The board, the tasks, the lab and inventory panels and the report tabs are wired end to end and your team has been running mornings on them for months. What V1 does to this module is not build it; it is *curate* it — the Marketing column, the "+3 vs yesterday" line and the dead link are gone — *correct* it, so the report money row agrees with Accounts, and *lock* it.

Of the four mobile gaps, one is urgent on its own merits: **the phone has been dropping Saturday from every Monday huddle**. That is the first item of the V1.1 mobile work, and it is worth knowing today: patients treated on Saturday have not been appearing on anyone's follow-up list on a phone. The other three are a money definition, a button that lies, and two removals that close themselves.

This was the right first chapter precisely because it asked so little of you: no configuration, no data migration, one permission to grant, and a ten-minute habit that your manager already owns. It is also, now, the proof that parity has to be read out of the code rather than clicked through — which is the more valuable finding.

2 Appointments

The book. Everything else in Dentfluence — the huddle, the recall list, the money — is downstream of what is written here, which is why an error in this module is never contained to this module.

IN V1

SIDEBAR → CLINICAL → APPOINTMENTS

MODULE PERMISSION: APPOINTMENTS

What it is

A day calendar with a live queue beside it. The calendar is the booking surface; the right-hand rail is the working surface — it holds today's counts by status and the patient queue, and it is where the front desk actually spends the day.

The important thing to understand about Appointments is that it is not a diary. It is the *system of record for whether a patient turned up*. Every status you click writes a timestamp, and those timestamps are what the recall engine, the huddle's Yesterday column, and the missed-appointment follow-up all read. A no-show that nobody marks is a patient nobody chases.

Why it exists

The business problem, stated plainly.

A paper book tells you who is coming. It does not tell you who came, who did not, who was moved, or who left without a next date. Those four facts are where the revenue is. The module exists to capture them as a by-product of the front desk doing its normal job — moving a patient from **Scheduled** to **Checked In** to **In Chair** to **Done** — rather than as a separate reporting chore that nobody would do.

Who uses it, and when

PERSON	WHEN	WHAT THEY DO
Front desk	All day	Book, reschedule, cancel with a reason, and advance every patient's status as they move through the clinic.
Doctors	Between patients	Read their own column. The scope switch flips between My Schedule and Whole Clinic .
Assistant	Morning and on the hour	Reads the queue to know which chair is next.
You	Never, directly	You read the consequences on the Huddle and in Reports.

What is on the screen

One page. Calendar left, working rail right.

TOP BAR

Search, scope, clock, two buttons

A patient search box; then either a doctor filter or the **My Schedule** / **Whole Clinic** pills depending on who is logged in; a live clock and today's date; and the two write actions — **Walk-In** and **Add Appointment** .

Note: a walk-in is a different kind of record, not just a late booking. It is excluded from the huddle's collections target on purpose, because nobody could have planned for it the night before.

CENTRE

The calendar

The day grid, drawn in the browser. Drag to move, click to open. Booking into an operatory opens a picker; cancelling opens a box that requires a reason.

Comes from: the appointments table for the selected day and doctor scope. Nothing else feeds it.

RAIL · TOP

Eight status counters

Total · Scheduled · Checked In · In Chair · Done · Cancelled · No Show · Walk-In. Each is today's count for that status.

Read this carefully: the chips do *not* add up to Total. There is a ninth status, **Checked Out** , which has no chip — a checked-out patient is inside Total and inside none of the eight. Do not use the chips as an arithmetic breakdown.

Patient Queue

Today's patients as cards, with an **N in queue** count above. This is the list the assistant works from.

Comes from: the same day list as the calendar, filtered in the browser. It refreshes itself every thirty seconds.

Where the numbers come from

FIGURE	WHAT IT ACTUALLY COUNTS	SCOPE
"N patients" (total pill)	Appointment <i>rows</i> for today — not distinct people. A patient booked twice in one day counts twice.	Today, current doctor scope
Scheduled / Checked In / In Chair / Done / Cancelled / No Show	A straight count of today's rows carrying that status.	Today, current doctor scope
Walk-In	Today's rows flagged as walk-in, whatever their status. This chip overlaps every other chip.	Today
N in queue	Counted in the browser from the visible list, not on the server.	Today, as filtered

ONE COUNTER BEHAVIOUR TO KNOW BEFORE YOU TRAIN ANYONE ON IT

When a doctor viewing **My Schedule** changes a patient's status, the counters that come back are **whole-clinic** counts, not that doctor's. The numbers above their own grid briefly describe someone else's day, until the thirty-second refresh corrects them.

This is a real defect, it is not on the V1 fix list, and it is harmless as long as nobody makes a decision from a counter in the second after clicking a status pill. Say that plainly in training rather than pretending the number is always right.

What it triggers elsewhere

Appointments is one of the most connected modules in the product. Every status change writes an entry to the patient's activity timeline, and one of them starts real work:

- **Marked No Show** → the rules engine creates an *urgent call task* for the front desk, with a three-day cooldown so the same patient cannot generate a task every day.
- **Booked, cancelled, checked in, completed, rescheduled, reverted, deleted** → each writes a timeline entry against the patient. Nothing else fires from these.
- **A consultation recorded against the appointment** → the appointment is closed to **Done** automatically. The doctor's work is what closes the booking, not a second click by reception.

What it can't do yet

1 · CHAIR UTILISATION IS CALCULATED AND NEVER SHOWN

The system works out how much of your chair capacity today's bookings consume — booked minutes, capacity minutes, a percentage — on every single page load, and then does not display it. The only page that shows it, **/appointments/today**, has no link anywhere in the product; you can only reach it by typing the address.

It is also working from a default it has never been told: fourteen hours of capacity a day, and a chair count guessed from whatever chair numbers appear in the data. Before that percentage means anything, those two numbers need to be your real ones.

2 · THE SIDEBAR BADGE CAN NEVER LIGHT UP

The Appointments entry in the sidebar has a badge, and it is fixed at zero. Zero badges are hidden, so nobody will ever see it. Harmless, but do not wait for it.

3 · THERE IS NO CONFIRMATION MESSAGE WHEN A BOOKING IS MADE

Booking an appointment sends the patient nothing. Reminders are a separate morning sweep, not a per-booking event. This is covered properly in **Chapter 8**, and it is the single most common thing people assume the system does and it does not.

Status

In V1, unchanged. No W-row touches Appointments. What you see on 14 September is what is running now, with the three limitations above intact.

3 Patients

The register, and the profile behind it. This is the only module every other module points at, which makes duplicate and merged records the most expensive kind of mess in the system.

IN V1

SIDEBAR → CLINICAL → PATIENTS

MODULE PERMISSION: PATIENTS

What it is

A searchable, filterable list of everyone the clinic has ever registered, and behind each row a profile with tabs for consultations, treatment plans, visits, documents, billing and the relationship timeline.

The list itself is deliberately plain — no dashboard, no stat cards, no aggregates. That is a design decision, not an omission: the moment you put clinic-wide figures on a data-entry screen, people start reading them instead of the row in front of them.

Why it exists

The business problem, stated plainly.

Every clinic accumulates two of the same patient. Two records means half a history, a recall that goes to the wrong number, and a balance that looks settled because the money is on the other record. Patients exists to be the one place a person is created, found, and — when it has already gone wrong — merged.

Who uses it, and when

PERSON	WHEN	WHAT THEY DO
Front desk	Every new patient	Register, search, correct details, link family members.

PERSON	WHEN	WHAT THEY DO
Doctors	Before and during every consultation	Read the profile — alerts, allergies, history, plans.
Assistant / Accounts	As needed	Read only. Neither can edit a patient record.

What is on the screen

The list, then the profile.

LIST Ten columns and a filter drawer Patient ID · Name · Age · Gender · Phone · Area · Source · Membership · Follow-up · Registered. Sort by newest, oldest, name, patient ID or last visit. The Filters drawer holds nine filters — gender, area, age range, membership, follow-up, source, birthday month and family — and the button carries a badge counting how many are active. The only number on this page is "N patients registered", which is the count of rows matching whatever filters are on. With no filters, it is your register size.
PROFILE HEADER Four money figures Total Billed · Total Collected with a percentage · Outstanding · and Advance, which appears only when the patient has credit on file. All four are lifetime, per patient, summed across every invoice that patient has ever had. They are not affected by any date filter anywhere in the product.

Where the key numbers come from

FIGURE	DEFINITION	BASIS
Total Billed	Sum of every invoice total for this patient.	Billed · lifetime
Total Collected	Sum of the paid amount on every invoice for this patient.	Collected · lifetime

FIGURE	DEFINITION	BASIS
Outstanding	Sum of the balance still due across their invoices.	Billed minus collected · lifetime
Advance	Money the patient has paid that is not yet applied to an invoice. It is a liability, not revenue.	Wallet balance

What it triggers elsewhere

- **Registering a patient** links them into the relationship spine, so the recall engine and the timeline can see them from day one.
- **A duplicate phone number** raises a soft warning that the front desk must acknowledge before saving. This is your first line of defence against the double-record problem, and it can be clicked through — so it is also a training point.
- **Opening a profile** writes an audit entry recording who viewed it. That is a DPDP requirement and it works.
- **Merging two patients** redirects the losing record to the survivor permanently.

What it can't do yet

1 · THE RECALL STATUS CARD ON EVERY PATIENT PROFILE IS DECORATIVE

Every patient shows **Recall Status: Up to Date** with a blank date, and a green **Active** pill beside their name. These are not calculated. The two fields behind them are never written by anything in the system — they hold the value they were created with, for every patient, forever.

The real recall engine ignores both of them and works off the last visit date instead, so **recall itself works** — see Chapter 8. It is only the display on this screen that is meaningless. Tell your team to ignore that card. It is the single most misleading thing on the patient profile.

2 · THE CONSULTATION TAB GIVES A DIFFERENT RECALL ANSWER

The Consultations tab on the same profile calculates its own recall status from days since the last visit, and its own due date as last visit plus six months. It agrees with nothing else. A patient can read **Overdue** on one tab and **Up to Date** in the header at the same time, on the same screen.

3 · NO AGGREGATE VIEW OF YOUR REGISTER

There is no screen that tells you how many active patients you have, how many came from each source this quarter, or how your register is growing. The Reports module has a Patients tab that partly covers this. The Patients module itself has none, by design.

Status

In V1, unchanged. No W-row touches Patients. Points 1 and 2 above are V1.1 items and should be named in staff training so nobody trusts the card.

4 Consultations & Treatment Plans

What the doctor found, and what the doctor proposed. These are two records and one conversation, which is why they share a chapter and share a tab.

IN V1

PATIENT PROFILE – CONSULTATION TAB · TREATMENT PLAN TAB

MODULE PERMISSION: PATIENTS

What it is

A consultation is the clinical encounter — complaint, findings, diagnosis, advice. A treatment plan is the priced proposal that follows it. Both live on the patient's profile; neither has a clinic-wide list screen, and that is deliberate.

There are seven kinds of consultation, five of which have their own button:

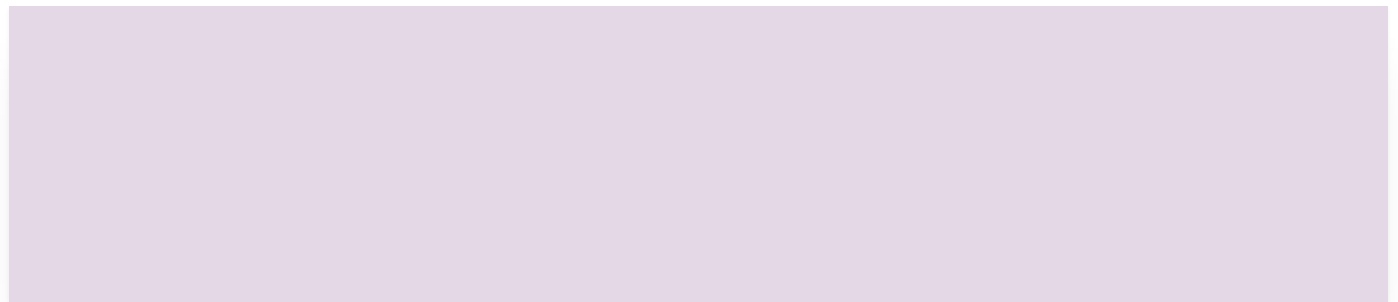
New Consultation , **Same Issue** , **Minor Visit** , **Emergency** and **COHA** . Each opens a form shaped for that encounter rather than one long form that fits none of them.

Why it exists

The business problem, stated plainly.

Case acceptance is the difference between a busy clinic and a profitable one, and it is decided in the ten minutes between the finding and the price. This pair of records exists so that the finding, the option, the price and the patient's answer are all written down in the same place at the same time — so that six weeks later you can answer "what did we actually offer them, and what did they say?"

What is on the screen



The record list

Five create buttons, then every consultation as a card — type, date, doctor, and per-card View / Print / Edit / Delete. Any linked prescription shows underneath with its drug count.

Comes from: this patient's consultations only. There is no clinic-wide consultations list anywhere in the product — it was removed on purpose because nothing linked to it.

Four support cards

Recall & Follow-up · **Patient Snapshot** (complaint, medical alerts, allergies, habits) · **Active Opportunities** · **Quick Actions** .

Do not trust the Recall card here — see Chapter 3. It calculates its own answer and disagrees with the profile header.

Building an option

Plan name, consultation, doctor, date, expected duration and visit count; then item rows — treatment, tooth, units, fee, discount, notes — with a dental chart picker and a running total at the foot.

Comes from: your treatment master list and its default prices.

The options, grouped by consultation

Each option shows its number, name, an **Accepted** or **Presented** badge, a progress chip, the estimated investment, the treatment count, and the item list. Then the four decision buttons: Mark as Accepted · Present Plan / Give Estimate · Defer · Reject.

Progress is derived, never stored. It is worked out from the clinical work actually recorded against the plan's items — which means it cannot be gamed by ticking a box.

Where the key number comes from

Estimated Investment on a plan card is the sum of its item totals, and each item total is calculated as: fee × units, less the item discount percentage, plus GST on the discounted figure.

THE RUNNING TOTAL WHILE YOU ARE TYPING DOES NOT INCLUDE DISCOUNTS OR GST

The grand total that updates live at the foot of the plan form multiplies fee by units and stops there. The figure that gets *saved*, and the one that appears on the plan card afterwards, applies the discount and adds GST.

Consequence: **if you use per-item discounts or GST, the number you quote from the screen while building the plan is not the number the patient will be shown.** Build the plan, save it, then quote from the card. This is a genuine defect, it is not on the V1 fix list, and it is a one-line habit to work around.

What it triggers elsewhere

- **Recording a consultation** advances the patient's last-visit date and *closes the linked appointment to Done*. The doctor's work closes the booking.
- **Marking a plan Presented** moves the opportunity to **Quoted** in the relationship pipeline.
- **Marking a plan Accepted** moves it to **Committed**, stamps the acceptance, writes a decision record, and writes the timeline entry.
- **Reverting an acceptance** requires a reason, is refused outright if the plan already has invoices, and is logged against the staff member who did it.

What it can't do yet

1 · THE PLAN LIFECYCLE STOPS AT "ALL WORK RECORDED", NEVER "COMPLETED"

This is deliberate and correct. The system will tell you every item on a plan has been carried out; it will not declare the case finished, because only a clinician can say that. Do not wait for a "Completed" state that is not coming.

2 · THERE IS A "SUGGEST A PLAN" FEATURE AND IT IS NOT CONNECTED TO ANYTHING

An automatic plan-suggestion endpoint exists in the code and no button anywhere calls it. It is worth knowing it exists only so that nobody switches it on believing it is intelligent: it is a short list of keyword matches to fixed treatment pairs, and it looks prices up by exact treatment name — anything that does not match exactly gets a price of zero. It should stay unreachable.

3 · NO CROSS-PATIENT VIEW OF PLANS

You cannot ask "show me every plan presented last month and whether it was accepted" from this module. The nearest thing is the opportunity pipeline in Chapter 8, which answers it approximately.

Status

In V1, unchanged. Case Acceptance — the separate patient-facing microsite that presents a plan to the patient on their own phone — was switched **off in production on 4 September** and is not part of V1. It is covered in Appendix B.

5 Treatment Visits

The clinical log. What was actually done, on which tooth, on which day — the record everything downstream counts as work.

IN V1

PATIENT PROFILE → VISITS TAB

MODULE PERMISSION: PATIENTS (EDIT TO RECORD)

What it is

A dated log of chairside work, one card per visit, each carrying the treatment, the tooth, the doctor, vitals, notes, any prescription, and the items of work recorded against a treatment plan.

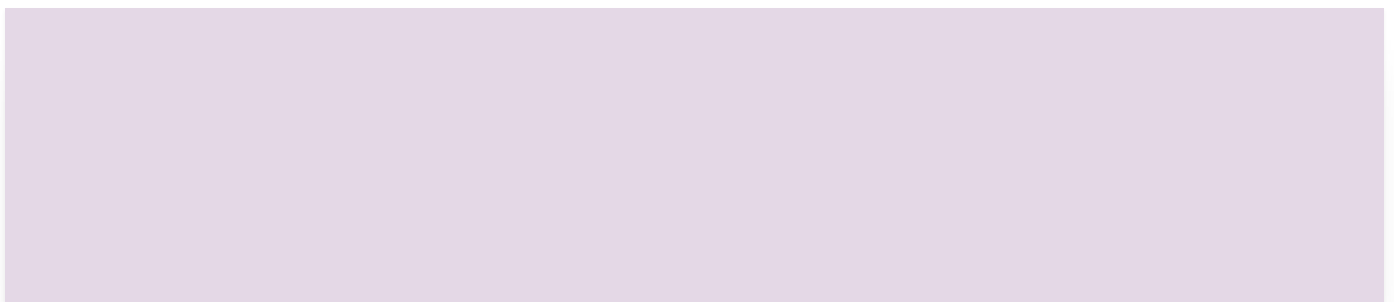
The visit is the most consequential record in the clinical stack. A treatment plan is a proposal; a visit is a fact. Plan progress, the six-month recall, the implant follow-up, the lab case and the front desk's billing prompt are all created from what is written here.

Why it exists

The business problem, stated plainly.

Work that is done and not recorded is work you cannot bill, cannot follow up, and cannot prove. The visit log exists so that recording the work — which the doctor has to do anyway — is what sets the rest of the clinic in motion, rather than the doctor also having to remember to tell reception to raise an invoice, and remember to book a recall, and remember to send the lab case.

What is on the screen



TAB

Visit Log

A search box and a list of visit cards, newest first. Each card: the date block, visit type, status, treatment, tooth, doctor, complaint, notes, the vitals row, the prescription count, the next-visit date and the item count.

There is no summary bar on this tab, and there used to be. It was removed on purpose: aggregate figures do not belong on a data-entry screen.

FORM

Recording a visit

A full page rather than a modal, because a visit has too many fields for a box. Vitals, work items against plan lines, work outcome per item, lab section, implant placement, next action.

What is not on this form: money. Cost and payment are deliberately excluded — see below.

Where the key numbers come from

The vitals — BP, pulse, SpO₂, temperature, blood sugar, weight — are typed and stored as typed. The counts are lengths of lists. The one number that needs explanation is the money line, and the explanation is that it should not be there.

THE MONEY LINE ON A VISIT CARD IS A LEGACY ARTEFACT — IGNORE IT

Some older visit cards show Rs. N , Paid Rs. N and Due Rs. N . Those fields **cannot be filled in from the visit form at all** — billing was deliberately moved out of the clinical record and given to the front desk through billing prompts. Any figure you see there came from an older import.

The line also disappears from a card as soon as anyone edits and saves that visit, and reappears on a page reload. That is a display defect, not data loss. It is not on the V1 fix list. **The billing truth is in Accounts, Chapter 6 — never read money off a visit card.**

What it triggers elsewhere

More than any other single action in the product. Saving one visit can set five things in motion, all inside one transaction, so either all of it happens or none of it does:

- **A billing prompt** is created for the front desk for every visit with work items. This is the handover that replaces the doctor telling reception what to charge.

- **A lab case** is created if the lab section was filled in.
- **A six-month recall task** is created if the visit marks the treatment complete — unless a matching recall task is already pending, so it cannot pile up.
- **Implant placement and stock deduction** are recorded together, once, if an implant was placed.
- **The treatment plan re-derives its own progress.** The visit never writes plan status directly, which is why progress cannot drift out of line with the clinical record.

What it can't do yet

1 · THE IMPLANT FOLLOW-UP ONLY FIRES ON THE EXACT WORD "IMPLANT"

The automatic implant follow-up task is created by matching the treatment name against the single word **Implant**. A treatment named "Implant Placement", "Dental Implant" or "implant" in lower case is classified as ordinary work and generates no implant follow-up.

This is a naming discipline problem with a real clinical cost. If you want the implant follow-up to work, the treatment in your master list must be named exactly **Implant**, or the follow-up must be booked by hand every time.

2 · THREE OLD STATUS VALUES CAN BE READ BUT NOT SAVED

Imported visits may carry **started**, **ongoing** or **abandoned**. The form will not accept those values, so editing such a visit forces a status change. Rare, and worth knowing when it happens.

3 · AD-HOC WORK IS INVISIBLE TO PLAN PROGRESS

Work recorded without linking it to a treatment plan line, and without a work outcome, writes no clinical activity entry. It is in the visit record, but plan progress and the timeline will not see it. If a plan looks stuck at "not started" while work is clearly being done, this is why.

Status

In V1, unchanged. No W-row touches Treatment Visits. The naming rule in point 1 is a clinic decision you can act on today at no development cost.

6 Accounts & Finance

The money screens. The longest chapter in this book, because this is where a wrong number costs you a decision rather than a click — and because more of this module changed in the run-up to 14 September than any other.

IN V1 · CORRECTED

SIDEBAR → OPERATIONS → ACCOUNTS & FINANCE

MODULE PERMISSION: FINANCE

What it is

One dashboard and six working pages behind it — Income, Expenses, Vendors, Membership, Wallets, Coupons and the CA Export. The dashboard is the read; the pages are the work.

Before anything else, one distinction that explains nine tenths of the confusion in this module: **billed** is what you invoiced, **collected** is what you actually received.

Dentfluence keeps them in two different places — invoices and payments — and every figure on every money screen is one or the other, never both. When two screens disagree, this is almost always why.

Why it exists

The business problem, stated plainly.

A dental practice does not fail from a lack of patients. It fails from money that was earned and not collected, and from an owner who could not tell the difference in time to do anything about it. This module exists so that at any moment you can answer three questions without opening a spreadsheet: what came in, what went out, and what is still owed.

Who uses it, and when

PERSON	WHEN	WHAT THEY DO
Front desk	All day	Raise invoices, take payments, work the billing prompts left by the doctors.
Accounts	Daily and at month end	Expenses, vendors, the CA export.
You	Daily, sixty seconds	The dashboard's top row. Nothing else.
Doctors	Never	Doctors have no Finance permission at all, by default. They cannot see any of this.

What is on the screen

The dashboard, top to bottom, as it will be on 14 September.

ROW 1

The four figures that matter

Collection · Expenses · Net Profit with a margin percentage · Outstanding with an invoice count.

These are your daily read. Three of the four follow the date filter at the top of the page. Outstanding does not — see below.

ROW 2

Three secondary figures

Today's Collection · Avg Daily Collection · Projected Period Total .

Reduced from five. Cash In Hand and Bank Balance have been removed for V1 — see the callout below.

ROW 3

Three charts

Revenue against expenses over the last six months; collection split by payment mode; your five biggest expense categories.

The six-month chart ignores the date filter — it is always the trailing six calendar months, by design.

ROW 4

Recent transactions and quick actions

The last ten payments received, whenever they were, and a set of shortcuts into the working pages.

Always the latest ten, regardless of the filter. It is a live feed, not a report.

FOOT

Income Tax Estimator

An indicative 44ADA presumptive calculation: projected annual revenue, taxable income at 50%, the slab, an annual liability and a per-quarter figure.

Labelled indicative on the screen, and it means it — see the honest limits below.

FILTER

The date control

Today · Yesterday · This Week · This Month · Quarter · Financial Year · custom. Defaults to month-to-date. The financial year is the Indian one, 1 April to 31 March.

Not everything obeys it. The table below says exactly what does and what does not.

Where every number comes from

The most useful table in this book. Read the last two columns before you read any figure.

FIGURE	WHAT IT IS	BILLED OR COLLECTED	FOLLOWS THE DATE FILTER?
Collection	Every payment received in the selected period.	Collected	Yes
Expenses	Every expense dated in the period — <i>including bills you have not paid yet.</i>	Recorded, not paid	Yes
Net Profit	Collection minus Expenses, exactly as above.	Mixed	Yes
Outstanding	Everything unpaid across every invoice	Billed	No — lifetime

FIGURE	WHAT IT IS	BILLED OR COLLECTED	FOLLOWS THE DATE FILTER?
	ever raised, as of right now.		
Today's Collection	Payments received today.	Collected	No — always today
Avg Daily Collection	Period collection divided by the days elapsed in the period.	Collected	Yes
Projected Period Total	The daily average multiplied out to the full period. A straight-line guess.	Collected	Yes
Collection by mode	The period's payments split by cash / card / UPI / transfer.	Collected	Yes
Top expenses	The period's five largest expense categories.	Recorded	Yes
Revenue vs expenses chart	Trailing six calendar months.	Collected vs recorded	No — fixed six months
Recent transactions	The last ten payments.	Collected	No — always latest ten

YOUR PROFIT DEFINITION — SETTLED 4 SEPTEMBER

Profit is income minus expenses. Nothing else. One definition, everywhere in the product, no adjustments, no accruals, no allocations.

What that means in practice, and you should know it: income is *collected* money, while expenses are *recorded* — a bill you have entered but not yet paid is already sitting in the expense figure. So the profit number is conservative in a month where you have entered supplier bills you have not settled. That is a reasonable place to be conservative, and it is a single, stable rule you can explain to your CA in one sentence.

REMOVED FOR V1 — TWO FIGURES THAT WERE DOING HARM

Cash In Hand is gone. It was never the money in your drawer. It was every cash payment ever received, minus every cash expense ever recorded, since the day the system started — floored at zero, ignoring the date filter, and never reset at day close. It only ever grew. A real cash-in-hand figure needs a cash drawer with an opening balance, a close and a count; that is V1.1 work and it was not going to be invented in a week.

Bank Balance is gone. It was the number zero, printed on your main money screen, with a note in the code saying "wire this up when balances are tracked".

Removing both was your call on 4 September and it was the right one. A missing figure prompts a question. A confidently wrong figure prompts a decision.

The working pages behind the dashboard

PAGE	WHAT IT IS FOR	WORTH KNOWING
Income	Invoices · Receipts · Final Bills · Trash	The four figures above the tabs — Today, This Week, This Month, Outstanding — are <i>fixed windows</i> . They ignore the date range you set for the tables underneath.
Expenses	All · Unpaid · Recurring · Voucher register	The Unpaid tab deliberately ignores the date range and shows everything outstanding. That is correct behaviour for a payables list.
Vendors	Supplier records and their bills	Feeds the payables side of Expenses.
Membership · Wallets · Coupons	Patient credit and schemes	Wallet money is a <i>liability</i> — a patient's advance is not your revenue until it is applied to an invoice.
CA Export	The pack you hand your accountant	Income in it is <i>collected</i> ; GST in it is <i>billed</i> . Tell your CA that, once, and it will save an argument.

How a payment actually lands

Two routes, and the difference matters when a patient hands over one amount against three invoices:

- **Against one invoice** — the settled amount is capped at that invoice's balance. This is the only route that handles card convenience fees and EMI.
- **Against the patient** — the money is allocated *oldest invoice first*, one receipt covering the whole tender, with a permanent record of how it was split. Anything left over after every invoice is settled goes to the patient's wallet as credit, not as revenue. Card and EMI are refused on this route.

An invoice's status is derived, not chosen: nothing paid is **draft**, part paid is **partial**, fully paid is **paid**. **There is no "send" step** — **draft** in this system means *unpaid*, not "not yet issued". Reception must not read it as "not finished".

What it can't do yet

1 · FIVE FINANCE PAGES EXIST WITH NO WAY TO REACH THEM

Finance Reports, **GST**, **Cashbook**, **Payroll** and the **Voucher register** are built and working, and nothing in the navigation links to any of them. You can only open them by typing the address.

This is worth a decision after 14 September: either link them, or accept that they are not part of how the clinic works. Leaving built pages unreachable is how a product accumulates the belief that it does less than it does.

2 · TWO "OUTSTANDING" FIGURES CAN LEGITIMATELY DISAGREE

The dashboard's Outstanding excludes cancelled invoices. The Billing list's own summary does not, and it is unfiltered and lifetime. If a cancelled invoice was cancelled by the older route, it keeps its balance and shows up in one place and not the other.

The dashboard figure is the one to trust.

3 · THE TAX ESTIMATOR ANNUALISES THE CURRENT MONTH, NOT YOUR FILTER

Whatever date range you set, the estimator takes what has been collected *this calendar month so far* and multiplies by twelve. On the 2nd of the month that is a projection built from two days. Its quarterly figure is simply the annual liability divided by four, which is not the schedule printed directly beneath it.

It is labelled indicative and it should be treated as a prompt to call your CA, not as a number.

4 · THE EXPENSE BASIS DIFFERS BETWEEN FINANCE AND ANALYTICS

The Finance dashboard counts expenses whether or not you have paid them. The Analytics screens count only expenses you have paid. The same month will show a different expense total, and a different profit, depending on which screen is open.

Finance is the canonical screen. Analytics is covered in Chapter 14 and carries its own health warning.

5 · THE CASHBOOK HAS NO OPENING BALANCE

Its running balance starts at zero every time the page loads. It is a period movement list, not a bank book. Related to the removal of Cash In Hand, and it resolves in the same V1.1 piece of work.

Your decisions

F1 The five unreachable Finance pages — link them or leave them?

WHAT IS REALLY HAPPENING

Five working pages have no navigation entry. Nobody uses them, so nobody has found their defects either.

MY RECOMMENDATION

Leave them out of V1 and decide in October, one page at a time, whether each earns a place in the sub-navigation. Adding five untested doors a week before training adds five ways to be surprised on the day.

STATUS
Open
V1.1

F2 Who is allowed to cancel an invoice?

WHAT IS REALLY HAPPENING

Invoice cancellation was hardened to admin-only on 4 September. Before that it had no guard at all behind a live button.

MY RECOMMENDATION

Keep it admin-only, meaning you. A cancelled invoice is the one action in the money stack that silently changes a total, and the clinic runs perfectly well with it going through you.

STATUS
Settled 4 Sep
Admin only

Status

In V1, corrected. Cash In Hand and Bank Balance removed; one profit definition applied; invoice cancellation locked to admin. Everything under "what it can't do yet" is accurate as at 14 September and none of it is being fixed before then.

7 Revenue by Treatment Category

The one new number in V1. It answers the question every owner asks and almost none can answer: which treatments are actually carrying this clinic?

NEW IN V1

FINANCE → REPORTS

MODULE PERMISSION: FINANCE

What it is

A table with one row per treatment category and three money columns: **Billed**, **Collected** and **Outstanding**. It is the only report in V1 that tells you where your revenue comes from rather than how much of it there is.

Why it exists

The business problem, stated plainly.

You already know your monthly collection. What you cannot currently see is that — for example — implants are twenty per cent of your billing and sixty per cent of your outstanding, or that scaling is a third of your appointments and a twentieth of your revenue. Those are the facts that change what you buy, what you train for, what you market, and which chair hours you protect. Without this table, every one of those decisions is made on impression.

Where the number comes from

Read this before you use the report for anything.

Revenue is attributed **by invoice line**. Each line on an invoice carries the treatment it was raised for, and that treatment's category is what the line is counted under. This is the right basis: it follows what was actually charged, item by item, rather than guessing a category from the appointment or from the plan.

COLUMN	DEFINITION	BASIS
Billed	The value of invoice lines in that category, raised within the period.	Billed · invoice date
Collected	Payment received and allocated against those lines.	Collected · payment date
Outstanding	Billed less collected — what that category is still owed.	Derived

THE TWO THINGS TO UNDERSTAND ABOUT THIS TABLE BEFORE YOU ACT ON IT

Billed and Collected are on different clocks. A crown billed in August and paid in September appears in August's Billed and September's Collected. Over a quarter the two converge; over a single week they will not, and that is correct, not an error.

Outstanding here is period outstanding, not the patient's whole balance. It answers "what has this category not been paid for out of what it billed in this period". The clinic-wide Outstanding on the Finance dashboard is a different, lifetime figure. The two are not meant to reconcile.

WHAT THIS REPLACES — AND WHY THE OLD VERSION WAS WORSE THAN NOTHING

A "revenue by treatment category" panel already existed on the Reports page. It read from a table that no part of the application ever writes to — the only things that ever put rows in it were the demo seeders. On a clean production database that panel is empty; on a database that still carries demo data it shows invented figures.

This is exactly the class of defect that the whole V1 exercise exists to remove: a screen that looks like a report and is not connected to your clinic.

How to actually use it

- 1 Monthly, not weekly.** A month is long enough for billing and collection to be roughly on the same clock.
- 2 Read Outstanding first, not Billed.** The category with the worst collection ratio is a front-desk or a consent problem, and it is fixable this week. The category with the highest billing is not a problem at all.

- 3 **Compare against chair time, in your head.** The table does not know how long a treatment takes. A category that is 10% of revenue and 40% of your chairs is the finding, and it will not be on the screen — you have to bring it.
- 4 **Do not compare doctors with it.** It cannot do that. Doctor-wise revenue was deliberately cut from V1 — see below.

What it can't do yet

DOCTOR-WISE REVENUE IS NOT IN V1

You asked for it. It was costed and cut on 4 September, and the reason is worth recording because it will come up again: invoice lines do not currently record *who performed the work*. Adding that means a schema change plus a backfill of historical invoices, and it was the single most expensive item on the list. It is a V1.1 item and it is genuinely wanted — it is deferred on cost, not on merit.

Until then, the honest answer to "which doctor earns what" is that the system cannot tell you, and you should not approximate it from this table.

Status

New in V1. This is the one addition rather than a correction. If the schedule tightens, this is the row most at risk — and if it slips, the honest position is that the old panel is removed and nothing replaces it until V1.1, which is still better than a report fed by demo data.

8 Relationships (PRE)

The chase list. The module most likely to be described as something it is not — so this chapter starts with what it does not do.

IN V1

SIDEBAR – COMMUNICATION – RELATIONSHIPS

MODULE PERMISSION: RELATIONSHIP

START HERE. THE SYSTEM DOES NOT CONTACT ANYBODY BY ITSELF.

There is no automated WhatsApp in your production system. There are no messaging credentials on the server, business verification with Meta has not been started, and until it is completed no template message can leave the system at all.

What PRE actually is: the system tells reception exactly who to chase, and gives them one button per patient. The button opens WhatsApp on the staff member's own phone with the message pre-written. A human still presses send.

Say it that way to your team, to your website, and to anyone who asks. Every other description of this module is a promise you cannot keep on 14 September.

What it is

A daily worklist of every patient who needs contacting, grouped by why, built fresh each morning from what actually happened in the clinic — plus the pipeline of leads and treatment opportunities behind it.

Why it exists

The business problem, stated plainly.

The revenue you have already earned is cheaper to collect than the revenue you have not. A patient who came six months ago, a plan that was approved and never booked, a lab case sitting back from the lab with nobody told, a post-operative check that nobody made — every one is money already half-earned and quietly abandoned. PRE exists to make sure that on any given morning, reception has a finite, ranked list of those people rather than a vague intention to "follow up".

What is on the screen

TAB 01

Today's Actions

The working screen. Eighteen categories in three bands: *Most Important* — today's confirmations, tomorrow morning's list, reminders, follow-up calls, patients treated yesterday, lab work ready, yesterday's missed appointments. *Leads & Opportunities* — new enquiries, lead follow-ups, treatment opportunities, membership renewals, yesterday's missed calls. *Other Reminders* — recalls, payment reminders, pending estimates, other calls, follow-up tasks, completed calls.

Comes from: about twelve different parts of the system, read live each time the page opens.

TABS 02–09

The rest

Dashboard · Pipeline (leads) · Opportunities · Recalls · WhatsApp · All Relationships · Analytics · Settings.

Today's Actions is the one that gets used. Everything else supports it.

What actually creates the work

The recall engine runs every morning at 07:00 and creates queue items on six triggers:

TRIGGER	RULE
No visit for six months	Last visit six months ago or more. One open item per patient, with a thirty-day cooldown.
Approved plan, no appointment	A plan the patient accepted and never booked. The most valuable row on the list.
Post-operative follow-up	Fourteen days after a surgical visit.
Lab work back, no appointment	The case is in the clinic and the patient does not have a date.
Recent treatment follow-up	Seven days after any treatment visit.
Birthday	Three days ahead.

Separately, a rescheduled call that was parked as "waiting for the patient" is returned to the pending list on its due date, at 06:45. That fix matters: before it existed, a call that reception postponed disappeared from the board permanently.

What can and cannot be sent

Seven message templates are configured. This is what each one actually does today.

TEMPLATE	REALITY IN PRODUCTION
Appointment reminder	Has an automatic send path, scheduled daily at 10:00. <i>Blocked by the missing credentials.</i>
Review request	Has an automatic send path, scheduled daily at 11:00. <i>Blocked by the missing credentials.</i>
Lab ready	Has a send path that fires when a lab case moves. <i>Blocked by the missing credentials.</i>
Appointment confirmation	Configured. Nothing in the system calls it.
Recall due	Configured. Nothing calls it. Recall is a call list.
Payment reminder	Configured. Nothing calls it. Overdue payments create a <i>task</i> , not a message.
Festive offer	Configured. Nothing calls it.
Birthday	Works today. It is a button that opens WhatsApp on the staff phone with the message written. A human sends it.

ONE BEHAVIOUR TO CORRECT IN TRAINING

Pressing the birthday button marks the row done on the board *whether or not the staff member actually sends the message*. The system cannot tell. If the board says done and the patient got nothing, this is how.

What it can't do yet

1 · THE RELATIONSHIP SCORE IS ALWAYS ZERO

Every relationship carries a score out of 100, shown as a column on the list, a badge on the profile, and a distribution chart in PRE Analytics. **It is never calculated.** Two independent faults stop it: one of its six factors looks for an appointment status that does not exist in this system, and another looks for a database table that was never created. The second throws an error which is caught and logged silently, so the score keeps its starting value of zero for every patient, forever.

Consequence: every patient is in the "Low" band on the analytics chart, and the Score column is a column of zeroes. **Tell your team to ignore it entirely.** It is not on the V1 fix list and it should be either fixed or hidden in V1.1 — a visible number that is always wrong is worse than no number.

2 · THE RECALL PERIODICITY SETTING DOES NOTHING

PRE Settings has a "General Recall periodicity" field, defaulted to 180 days. Changing it does not change the six-month cutoff, which is fixed in the code. The field is display only.

3 · THE SIDEBAR SHOWS RELATIONSHIPS TO EVERYONE

The link is visible regardless of permission, but the page behind it is gated. A staff member without the Relationship permission sees the link and gets a refusal. Cosmetic, and worth mentioning in training so it does not get reported as a fault.

Your decisions

R1 Start Meta business verification now, or after launch?		
WHAT IS REALLY HAPPENING	MY RECOMMENDATION	STATUS
Automated WhatsApp needs Meta business verification, then template approval at 24–48 hours per template, seven templates. None of it has started. It is the only item on the whole V1 list where the waiting is someone else's clock, not ours.	Start it this week, before launch. It costs no development days, it blocks nothing, and every day it is not started is a day added to the front of V1.1. It is the highest-return hour you can spend before 14 September.	Open Zero dev days

Status

In V1 as a worklist. The lists are correct and complete and reception has one button per patient. Nothing sends itself. Do not describe it otherwise on the website, on Instagram, or in the training room.

9 Lab

Nine states, one vendor relationship, and the only module in the product where a remake has a visible price tag.

IN V1

SIDEBAR → OPERATIONS → LAB

MODULE PERMISSION: LAB

What it is

A case tracker. Every impression or scan that leaves the clinic becomes a case that moves through nine states — draft, order placed, impression sent, scan sent, trial received, trial returned, final received, complete, rejected — with an expected date, a vendor, an estimated cost and an actual cost.

Why it exists

The business problem, stated plainly.

Lab work is the one thing in a dental practice that leaves the building and has to come back. Every day it is late is a chair hour you cannot sell and a patient losing confidence. And a remake is the purest form of loss in the practice — you pay twice, you occupy the chair twice, and you charge once. This module exists so that both of those are visible while you can still do something about them, rather than at the end of the month.

What is on the screen

TOP

Four money cards

This Month — Estimated and **Actual**, with the variance between them · **Unbilled Cases** with a count · **Billed, pending payment**.

The variance line is the point of this strip. Estimated against actual, this month, is the number that tells you whether your lab pricing is understood.

MIDDLE

Nine filter tabs and a search

All · Active · Draft · Order Placed · Impression/Scan · Trial Loop · Final Received · Complete · Rejected, each with a live count.

Active means everything that is not complete or rejected. It is the tab to live on.

TABLE

Eight columns

Patient · Work Type · Tooth/Shade · Lab Vendor · Expected · Status & Next Step · Est. Cost · Actions.

"Status & Next Step" is the useful one — it names what has to happen next rather than only where the case is.

ALSO

Dashboard, reconciliation, vendors

A lab dashboard, a monthly reconciliation against what the vendor invoiced, and the vendor records themselves.

Reconciliation is where the remake argument gets settled — with a record rather than a memory.

Where the numbers come from

FIGURE	DEFINITION
This Month — Estimated	Sum of the estimated cost of cases <i>created</i> this calendar month.
This Month — Actual	Sum of the actual lab cost on those same cases.
Unbilled	Cases that have an actual cost recorded and have not yet been billed to the clinic by the lab.
Billed	Cases the lab has invoiced and you have not yet paid.
Tab counts	A straight count of cases in each state.

Note on the month: "this month" is by the date the case was *created*, not the date it came back or the date it was billed. For a case that spans a month end, the estimate and the actual sit in the month it started.

What it triggers elsewhere

- **Every morning at 09:00**, every overdue case that does not already have an open task gets one. Trial cases that have sat two days waiting on a doctor's decision get a review task.
- **Changing a case's status** writes to the lab's communication thread and opens a queue item if none exists, then closes it automatically when the case reaches final received, complete or rejected.
- **A case arriving back** puts the patient on the recall engine's list if they have no appointment — see Chapter 8.
- **Recording lab work on a treatment visit** creates the case. Reception does not create most cases; the chair does.

What it can't do yet

1 · CREATING A LAB CASE NOTIFIES NOBODY

A case created and left in draft produces no signal to anyone until either someone changes its status or it becomes overdue at the next 09:00 run. There is no "new case" alert.

The practical consequence: a case created on Friday afternoon and forgotten is invisible until it is already late. The mitigation is a habit, not a feature — the Lab column on the huddle board is read every morning.

2 · AN OVERDUE COUNT IS CALCULATED AND HAS NO TAB

The system counts overdue cases and there is nowhere to click to see them. Use the Expected column with Active selected instead.

3 · LAB HAS NO SEPARATE EDIT PERMISSION

Anyone with Lab *view* can also change a case's status and archive a case. Unlike Inventory and Prescriptions, this module has no per-action permission. Under your default roles that is Admin, Manager and Doctor — which is acceptable — but it is worth knowing that "view only" is not available for Lab.

Status

In **V1**, **unchanged**. The permission point above is a real gap and it is a V1.1 item.

10 Inventory

The most instrumented module in the product, and the one with the largest gap between what it calculates and what it tells anyone.

IN V1

SIDEBAR → OPERATIONS → INVENTORY

MODULE PERMISSION: INVENTORY

What it is

Stock, purchase orders, goods receipt, implants, reusable assets and a stock count, with a dashboard on top that scores the whole thing out of a hundred.

Why it exists

The business problem, stated plainly.

Two failures, opposite in shape, identical in cost. You run out of a material mid-procedure and lose the patient's confidence and the chair hour. Or you hold three lakh of stock, a third of it expiring, because nobody could see what was already on the shelf. Inventory exists to keep both numbers visible at once — what is about to run out, and what is quietly dying in the cupboard.

What is on the screen

HERO

Inventory Health Score

A single score out of 100 with a letter grade, built by starting at 100 and deducting for out-of-stock, low-stock, expiring and dead stock, each capped.

Structurally optimistic — items with no minimum quantity set are counted in the denominator but can never trigger a deduction. Read it as a trend, never as a measurement.

KPI ROW

Six cards

Inventory Value · **Total Items** · **Low Stock** · **Out of Stock** · **Expiring Soon** (90 days) · **Assets Due** .

Inventory Value is available quantity × average purchase price, across active items.

SECOND ROW

Five more

Days Until Stock-out · **Dead Stock Value** (no movement in 90 days) · **Monthly Wastage** (expired plus damaged) · **Today's Deliveries** · implant catalogue count.

Days Until Stock-out is derived from the last thirty days' consumption. It is the most decision-useful figure on the page.

BELOW

Charts, lists, movements

Stock status donut, category values, a value trend, critical items, expiring soon, and the last twelve stock movements.

The "value trend" is not inventory value — it plots purchases per week for eight weeks. Read it as spending, not as holding.

How an item is classified

STATE	RULE
Minimum not set	No minimum quantity on the item. <i>Excluded from low, critical and out counts entirely.</i>
Out of stock	Nothing on hand.
Critical	On hand is at or below half the minimum.
Low	On hand is at or below the minimum.
In stock	Everything else.

THE SINGLE MOST VALUABLE HOUR YOU CAN SPEND IN THIS MODULE

An item with no minimum quantity is invisible to every alert, every count and the health score. It cannot be low, cannot be critical, cannot be out — it simply is not counted.

Set a minimum quantity on every item you actually use. Until that is done, Inventory is reporting on a subset of your stock and the health score is flattering you. This is a data job for your assistant, not a development job, and it is worth doing before 14 September.

What it triggers elsewhere

- **Creating a purchase order** creates two tasks automatically — confirm the order with the vendor, and a delivery follow-up the day before it is expected.
- **Receiving goods** creates an unpaid expense in Finance and closes the delivery follow-up task by itself.
- **An implant placed at the chair** deducts from stock as part of the visit record, once, without a separate step.

What it can't do yet

1 · STOCK CROSSING ITS REORDER LEVEL TRIGGERS NOTHING AT ALL

This is the most important sentence in the chapter. There is no alert, no notification, no task, no email, no scheduled check. The low-stock and out-of-stock numbers are calculated *only at the moment someone opens a page that shows them*.

Inventory is a pull system, not a push system. If nobody opens the Inventory Alerts page — or reads the Inventory column on the huddle board — nobody finds out. That column on the morning board is not a convenience; on current behaviour it is the entire alerting mechanism.

2 · TWO DIFFERENT DEFINITIONS OF "LOW"

The Alerts page classifies by *minimum quantity*. The create-purchase-order screen classifies by *reorder level*, falling back to minimum quantity. The two screens can disagree about which items are low. Set both fields to the same value per item and the problem disappears.

3 · TWO STOCK LIST PAGES, ONE OF THEM UNPAGINATED

There is a stock listing that has no navigation entry and returns every item at every location in one page. The sub-navigation's **Items** points at a different, proper page. Use the sub-navigation and ignore the other one.

4 · "ADD ITEM" ON THE DASHBOARD DOES NOT ADD AN ITEM

The quick action links to the read-only stock list. Add items from the sub-navigation instead.

Your decisions

11 Who owns the daily look at Inventory Alerts?

WHAT IS REALLY HAPPENING

Nothing pushes a stock alert to anyone. If no named person opens the page, running out of something is discovered at the chair.

MY RECOMMENDATION

Name the assistant, and make it a huddle step. The Inventory column is already on the morning board — the decision is only that one person is accountable for reading it and saying the word out loud. Zero development cost, and it closes the gap completely.

STATUS

Open
Training item

Status

In V1, unchanged. Points 1 and 2 are behaviour to train around, not defects being fixed before 14 September.

11

Prescriptions & Clinical Library

What you gave the patient, and what you hold about them. Two modules, one chapter, because they share a single question: who is allowed to see a patient's clinical material?

IN V1

SIDEBAR → CLINICAL → PRESCRIPTIONS · CLINICAL LIBRARY

MODULE PERMISSIONS: PRESCRIPTIONS · CLINICAL LIBRARY

What it is

Prescriptions is a proper prescribing module — a drug master, a form, an Rx number, a printable output and a WhatsApp send button. The Clinical Library is two different things sharing a name: the clinic's own content library, and the per-patient document store on the profile's **Documents** tab.

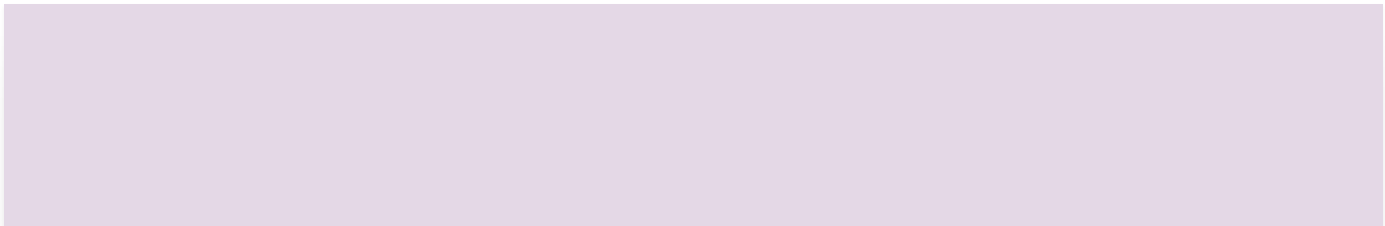
The one your team uses daily is the Documents tab: photographs, X-rays, OPGs, CBCTs, scans, PDFs, consent forms, lab slips.

Why it exists

The business problem, stated plainly.

Two reasons, and only one of them is clinical. The clinical reason is obvious. The other is that a dental practice is now a data fiduciary under the DPDP Act, and photographs of a patient's mouth are personal data. Where those files live, who can open them, and whether that opening is recorded, is a legal question as much as a clinical one.

What is on the screen



PRESCRIPTIONS

Four cards, then the list

Total · **Today** · **Issued** · **Draft** , then a filterable table: Rx Number, Patient, Date, Prescribed By, Drugs, Status.

The **Draft** card will read **zero, permanently**. There is no draft step — a prescription is live the moment it is saved. Ignore that card.

DOCUMENTS TAB

Sub-tabs with counts, then gallery or timeline

All · **Photos/Video** · **X-ray group** (X-ray, OPG, CBCT, STL, intraoral scan) · **Documents** · **Consent** , each with a count, plus a filter bar and two views.

Every count is of this patient's files only. There is no clinic-wide file count anywhere.

Where the numbers come from

All of them are simple counts, with one inconsistency worth knowing: on the Prescriptions page, **Total** includes deleted prescriptions while **Today** , **Issued** and **Draft** do not. The four cards therefore describe two slightly different populations. None of the four is branch-scoped.

What it triggers elsewhere

- **A prescription written at the chair** is linked to the visit and appears on the visit card with its drug count.
- **Downloading a clinical file** writes an audit entry. *Viewing one inline does not.*
- **Nothing else.** Neither module fires a task, a message or a rule.

What it can't do yet

1 · CLINICAL FILE PERMISSIONS ARE THE ONE OPEN SECURITY QUESTION IN THIS BOOK

The routes that list, upload, edit and delete a patient's clinical files are registered twice in the system, in two different files. One registration requires the Patients permission; the other requires only that you are logged in. The second one is loaded later and wins.

As things stand, any logged-in staff member can reach a patient's clinical file records regardless of their Patients permission. The image files themselves are protected by a separate branch check, so this is not an open door to the outside world — it is a door between roles inside your own clinic. With six staff in one branch, the practical exposure today is small. As a permission model it is wrong.

This is **C2** on your decision list and I need to know whether it is being closed before 14 September — see the decision card below.

2 · DOCUMENT FILTERING HAPPENS IN THE BROWSER

Every one of a patient's files is loaded into the page and then hidden by the filter. For a patient with a long history and large scans, that page will be slow. It is correct, just not fast.

3 · THE "REVISED" PRESCRIPTION STATUS IS NEVER USED

It exists and nothing writes it. A corrected prescription is a new one.

Your decisions

c2 Clinical file permissions — close before launch, or accept and date it?		
WHAT IS REALLY HAPPENING	MY RECOMMENDATION	STATUS
A duplicate route registration strips the Patients permission from clinical file actions. It is a small, well-understood change — delete one duplicate registration — but it is a code change, and this document phase carries zero development days.	Close it before 14 September if it fits in the W-1 batch; otherwise date it to the week after launch and say so here. What I will not do is write a handbook that describes a permission the system does not enforce. Tell me which, and this chapter states it plainly either way.	Open Needs your answer

Status

In V1. Prescriptions is unchanged and sound. The clinical file permission question above is the one unresolved item in this chapter.

12 Tasks & Practice Protocols

Where the clinic's loose ends collect, and where its routines are enforced. One is a to-do list nine other modules write into; the other is the closest thing the product has to an accountability sheet.

IN V1

SIDEBAR → OPERATIONS → TASKS · PRACTICE PROTOCOLS

MODULE PERMISSIONS: TASKS · PRACTICE PROTOCOLS

What it is

Tasks is a single list of everything owed by anyone, with four counters above it. Most of its contents are not typed by a human — nine parts of Dentfluence create tasks automatically, and that is the module's real value.

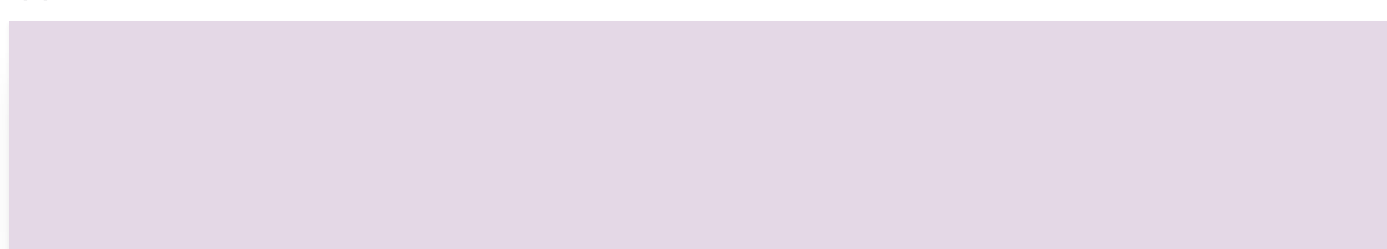
Practice Protocols is the recurring side: the standing routines each role owes, defined once and generated as real tasks every night at ten past midnight.

Why it exists

The business problem, stated plainly.

Two different failures. The first is the thing that fell between two people and neither picked up — that is Tasks. The second is the thing everybody knows should happen every Tuesday and nobody can prove happened — autoclave logs, stock checks, end-of-day cash reconciliation. That is Protocols, and it is the only place in the product that will tell you, person by person, what was assigned and what was actually done.

What is on the screen



TASKS · TOP

Four counters

[Overdue](#) · [Due Today](#) · [Upcoming](#) · [Completed](#) .

The **Completed** card is captioned "this period" and has no period. It is every completed task ever, for the branch. Read it as a lifetime total or ignore it.

TASKS · BODY

Three tabs

[Dashboard](#) — Backlog, Due Today, Upcoming, Completed sections · [My Tasks](#) · [Assign Task](#) .

Assistants, front desk and accounts see only their own tasks. Managers and doctors see the team's.

PROTOCOLS

The catalogue, grouped by role

Per protocol: title, frequency (once, daily, weekly, monthly), whether evidence is required, materials, and active state.

This screen is the definition, not the record. What actually got done appears in Tasks, and the scoreboard is on the Huddle's report tabs.

THE SCOREBOARD

Protocol compliance

Person by person: assigned, done, missed, percentage. It lives on the Huddle report tabs, not here.

The most decision-grade number in the product. It is about behaviour, and it is measured rather than reported.

What creates tasks automatically

SOURCE	WHEN
Practice protocols	Every night at 00:10, for the next day, to each assigned person.
Lab	09:00 daily — overdue cases, and trials waiting two days on a doctor.
Purchase orders	Two per order: confirm with vendor, and delivery follow-up.
Missed appointments	An urgent call task, with a three-day cooldown per patient.
Treatment completion	A six-month recall task, unless one is already pending.

SOURCE	WHEN
Overdue payments	A follow-up task. <i>Not a message.</i>
Shift reminders	Every five minutes.
Escalation	Every thirty minutes, tasks past their service level are escalated.

What it can't do yet

1 · FOUR CONTROLS ON THE TASKS PAGE DO NOTHING

The search box, the staff filter, the Daily/Weekly/Monthly buttons, and clicking a counter card to filter the list — none of them have any effect. They are drawn and connected to nothing. The buttons highlight, the list does not change.

This is the most visible cosmetic defect in the product, your team *will* find it in the first week, and it is not on the V1 fix list. **Say so in training rather than letting them report it four times.** Use the section headings — Backlog, Due Today, Upcoming — which do work.

2 · THE TASKS LIST IS NOT PAGINATED

Every open and completed task for the branch loads in one page. As the completed pile grows, this page gets slower. It will be fine on 14 September and it will need attention by January.

3 · THE OVERDUE PAGE IS AN EMPTY SHELL

`/tasks/overdue` renders a heading and nothing else. Nothing in the navigation points at it. Use the Overdue counter's section on the dashboard tab.

4 · A ONCE-ONLY PROTOCOL IS RE-EVALUATED EVERY NIGHT FOREVER

It only ever generates one task, because of a duplicate guard — but it is checked every night indefinitely. Harmless; worth knowing before someone asks why a one-off protocol never disappears from the catalogue.

Status

In V1, unchanged. Point 1 is the single most important thing to name in staff training from this chapter.

13 HR

Staff records, attendance and roles. The smallest module in daily use, and the one carrying the sharpest open question in this book.

IN V1

SIDEBAR → OPERATIONS → HR

MODULE PERMISSION: HR

What it is

Staff profiles, a daily attendance register, training records, memos, a calendar, and — behind a separate admin-only door — the roles and permissions screen.

Why it exists

The business problem, stated plainly.

Attendance in a small clinic is usually a register on the desk or nothing at all, and it becomes a problem exactly once — at an argument about a salary, months later, when neither side has a record. HR exists so that the record is made daily, as a thirty-second act, and is there when it is needed.

What is on the screen

ATTENDANCE

A day at a time

A date picker, a department filter, a **Mark All Present** button, then a row per staff member: shift, status, check-in, check-out, method.

Six statuses: present, absent, late, half day, on leave, holiday. Bulk marking allows only present, absent and holiday.

ABOVE IT

Four counts

Present (present, late and half day together) · **Absent** · **On Leave** · **Not Marked** .

Not Marked is total staff minus rows recorded, and it can go negative if a record exists for someone without an HR profile.

What it triggers elsewhere

- **Weekdays at 10:30**, anyone with no attendance row for the day is marked absent automatically, with a note saying the system did it.
- **Nothing else.** No leave accrual, no payroll run, no hours calculation, no notification when a professional licence is about to expire — the expiring-licence list on the HR dashboard is a display only.

TWO THINGS ABOUT THE AUTO-ABSENT JOB

It runs **weekdays only**. If your clinic works Saturdays, Saturday is never auto-marked and must be marked by hand or it stays blank.

Rows it creates are labelled method **manual** , so the Method column reads "manual" for rows no human touched. Do not read that column as proof anyone was present.

What it can't do yet

1 · THE QR ATTENDANCE PAGE IS OPEN TO ANYONE, AND IT LISTS YOUR STAFF AND THEIR CODES

There is a staff QR check-in screen that sits *outside* the login. Opening it does not require an account, and the page it returns contains every active staff member's name, role, and the personal code that the check-in and check-out actions accept.

In plain terms: anyone who can reach that address can see your staff list and mark any of them in or out. The only guard on the action is that the same person cannot be marked twice within five minutes.

This is not on any V1 row and I did not know about it before writing this chapter. It needs a decision from you this week — see the card below.

2 · HOURS ARE STORED AND NEVER CALCULATED

Check-in and check-out times are recorded. Nothing anywhere works out hours worked or overtime. Attendance answers "were they here", not "how long".

3 · EVERY NON-ADMIN ROLE CAN EDIT HR BY DEFAULT

Under the seeded permissions, Manager, Doctor, Assistant, Front Desk and Accounts all have HR view *and* edit. An assistant can mark anyone's attendance. Roles and permissions themselves are separately locked to admin, so nobody can promote themselves — but the attendance register is open to the whole team as shipped.

4 · ATTENDANCE HAS NO SEPARATE EDIT PERMISSION

HR view is enough to mark attendance. There is no view-only HR role.

Your decisions

H5 The unauthenticated QR attendance page — disable, restrict, or accept?

WHAT IS REALLY HAPPENING

A pre-login page exposes the staff roster and the codes that mark attendance.

Whether anyone outside the clinic could find the address is a separate question from whether they should be able to use it.

MY RECOMMENDATION

Disable it before 14 September and mark attendance from inside the app.

You are not using QR check-in today, the manual register works, and removing a public page is a smaller change than securing one. If you do want QR check-in later, it should be built properly in V1.1.

STATUS

Open

Needs your answer

H6 Should the whole team be able to edit HR attendance?

WHAT IS REALLY HAPPENING

The seeded permissions give every role HR edit. That is a settings change, not a code change — you can fix it yourself in Settings → Staff & Roles.

MY RECOMMENDATION

Reduce Doctor, Assistant and Accounts to HR view. Leave Manager and Front Desk with edit, because one of them marks the register each morning. Five minutes in Settings, zero development cost, and it makes the register mean something.

STATUS

Open
Settings change

Status

In V1 as a manual register. HR is also the module with no mobile support at all — there is no interface behind it for a phone, which is why HR attendance is web work before it can ever be a V1.1 phone feature.

14 Reports & Analytics

Three reporting surfaces, three different default date windows, and one rule for using them safely.

IN V1

SIDEBAR → INSIGHTS → REPORTS · ANALYTICS

MODULE PERMISSIONS: REPORTS · ANALYTICS

What it is

Three separate things that all look like reporting. **Reports** is the clinic-wide one with six tabs. **Analytics** is a set of eight deeper finance-oriented screens. And **Finance Reports** is a twelve-tab accounting pack that has no link anywhere in the navigation.

Why it exists

The business problem, stated plainly.

Because the daily screens answer "what is happening" and none of them answer "is this better or worse than last month". That second question is the only one that changes what you do.

The rule for using any of them

CHECK THE DATE WINDOW BEFORE YOU READ A SINGLE NUMBER

Reports defaults to **the last 30 days**. The Finance dashboard defaults to **month to date**. Analytics defaults to **the last 6 or 12 months** depending on the screen. Finance Reports defaults to **the financial year**.

Four surfaces, four defaults. Two of them will disagree about "this month" purely because one of them is not showing this month. Almost every "the numbers don't match" question in this product resolves to this.

What is on the screen

REPORTS

Six tabs

Appointments · Revenue · Patients · Treatments · Lab · Inventory. Windows of 7, 30, 90 or 365 days, or custom.

This is the surface to teach. One door, six subjects, a date control that works.

ANALYTICS

Eight cards

Vendor · Expense · Lab · Procurement · Cash Flow · Outstanding · Business Intelligence · Audit & History.

Admin only under the default permissions. Nobody else can open it.

Where the Revenue tab's numbers come from

FIGURE	DEFINITION	BASIS	PERIOD
Collected	Payments received in the window.	Collected	Follows the window
Outstanding	Everything unpaid across all invoices.	Billed	Lifetime
Invoice count / average	Invoices raised in the window, cancelled excluded.	Billed	Follows the window
Collection rate	Collected divided by collected plus outstanding.	Mixes a period figure with a lifetime one — see below.	
Top day, by mode, top patients	Payments in the window.	Collected	Follows the window

What it can't do yet

1 · "COLLECTION RATE" IS NOT A RATE

It divides what you collected in the selected window by that plus everything ever outstanding. Over seven days it will look catastrophic and mean nothing. **Ignore this figure.** Read Collected and Outstanding separately.

2 · ANALYTICS → AUDIT & HISTORY TOTALS ARE NOT TOTALS

The summary figures on that screen add up only the rows fetched for display — the fifty most recent payments, fifty vouchers, twenty reconciliations — not everything in the period. They are labelled as summaries and they are samples. Do not quote them.

3 · TWO ANALYTICS TABS IGNORE THE DATE FILTER THEY DISPLAY

Receivables and Payables in the Finance Reports pack accept a date range and do not apply it. They always show everything. That is defensible for a payables list and confusing next to a date control.

4 · THE REPORTS PAGE IS STILL TITLED FOR APPOINTMENTS

A leftover from when it only did one thing. Cosmetic.

Status

In **V1, unchanged**, with one correction: the duplicate Analytics door was removed on 4 September. Analytics previously had two addresses, one of which let a finance-only role past the Analytics permission. There is now one door.

15 Roles, Permissions and the Activity Log

Who can do what, and what gets written down when they do it. You asked for this in V1 and it is the only structural change in the release.

CORRECTED IN V1

SETTINGS → STAFF & ROLES · SETTINGS → AUDIT LOGS

ADMIN ONLY

What it is

Six roles, twenty modules, and three permission levels — view, edit, delete — set for each combination. Plus an audit log recording who did what.

Your eight job titles map onto the six roles the system has. There is no separate consultant-dentist scope and no communications role; both are V1.1.

YOUR JOB TITLE	ROLE IN THE SYSTEM
Owner	Admin
Clinic manager	Manager
Front desk	Front Desk
Associate dentist	Doctor
Consultant dentist	Doctor — no separate scope exists
Assistant	Assistant
Finance manager	Accounts
Comms team	No role exists. Use Front Desk.

The permission matrix as shipped

V = view, E = edit, D = delete. A dash means no access at all. This is the seeded default — Settings can change any of it.

MODULE	ADMIN	MANAGER	DOCTOR	ASSISTANT	FRONT DESK	ACCOUNTS
Dashboard	VED	—	—	—	—	—
Daily Huddle	VED	VE	VE	—	—	—
Patients	VED	VE	VE	V	VE	V
Appointments	VED	VE	VE	V	VE	—
Treatments	VED	VE	VED	—	—	—
Clinical Library	VED	VE	V	V	VE	V
Prescriptions	VED	VE	VE	V	VE	V
Relationships (PRE)	VED	VE	VE	VE	VE	VE
Accounts & Finance	VED	VE	—	—	VE	VE
Inventory	VED	VE	—	—	—	—
Lab	VED	VE	VE	—	—	—
Tasks	VED	VE	VE	V	V	—
Practice Protocols	VED	VED	—	—	—	—
HR	VED	VE	VE	VE	VE	VE
Reports	VED	V	—	—	—	V
Analytics	VED	—	—	—	—	—
Settings	VED	—	—	—	—	—
Marketing	VED	VE	—	—	—	—

Three things worth noticing in that table. **Doctors cannot see money at all** — no Finance row. **Everyone can edit HR** — see Chapter 13, and change it. And **only you have Settings, Analytics and the Dashboard**.

The change you asked for

Why this was worth a V1 row rather than a V1.1 one.

Until this release, the permission check on most screens proved only that you were allowed to *look* at a module. The same check was then reused on the buttons that *change* things. Setting a role to "view only" on a module therefore did not reliably stop that role writing to it.

Under the default six roles this was mostly masked, because view and edit are granted together almost everywhere. It became real the moment you did the sensible thing — gave someone view and withheld edit. That is the situation this row fixes, and it is why it was worth spending a day and a half on before letting six people loose on the system.

The second half of the row is the log: **every change to a role or a permission, and every money-write, is recorded** — who, what, when. That is the part that makes the permissions worth having, because a permission you cannot audit is a policy, not a control.

Where to read the log

[Settings](#) → [Audit Logs](#) . Admin only. It reads the audit and staff-activity records — logins, views, edits, deletes, and the role and money events added in this release.

What it can't do yet

1 · THERE IS NO CLINIC-WIDE ACTIVITY FEED

Separately from the audit log there is a patient timeline — every appointment, every plan decision, every recall — but it can only be read *one patient at a time* from their profile. There is no screen that answers "what happened across the clinic yesterday" from that timeline. The audit log answers the compliance version of that question; the clinical version has no screen.

2 · A FAILED LOG ENTRY IS SILENT

The timeline is deliberately built so that it can never break the action it is recording. If a timeline write fails, the action still succeeds and a warning goes to the server log. That is the right trade — but it means the timeline is a very good record and not a guaranteed one. The audit log is the stronger of the two.

3 · THE "DASHBOARD" PERMISSION DOES NOTHING

It appears in Settings and no screen checks it. Turning it off changes nothing.

4 · HELP IS HIDDEN FROM ALMOST EVERYONE

The Help page is open to every logged-in user, and its only link sits inside the admin-only Settings section. Five of your six roles cannot find it. Worth a link on the sidebar in V1.1.

Status

Corrected in V1. This is the one thing in the release that is new capability rather than removal of something false — and it is the reason a six-person team can be given six different logins on 14 September rather than sharing yours.

16 Settings & Compliance (DPDP)

The room only you go into. Everything here is admin-only, and after 14 September that is enforced rather than assumed.

CORRECTED IN V1

SIDEBAR → SYSTEM

ADMIN ONLY

What it is

Twenty-one settings panels in seven groups, and four compliance screens that exist because of the Digital Personal Data Protection Act.

Settings, by group

GROUP	WHAT IS IN IT
Organization	Clinic profile · Operatories · Health Facility registration
People & Access	Staff & Roles — the matrix in Chapter 15
Connections	Banking
Platform	Audit Logs
Advanced	Feature flags — see Appendix B
Data	Import / Export
Legacy	Thirteen panels being moved into their own modules over time: masters, knowledge bank, clinical, patient defaults, clinical library, billing & invoice, printing, notifications, growth & comms, inventory, huddle, personalisation, calendar

Two things the settings search offers and the product does not have:

Working Hours and **Appointment Slot Duration** . Both are marked "coming soon" and are not clickable. They are the only two advertised-but-absent settings; everything else in the search resolves to a real panel.

Compliance — the four DPDP screens

01

Consent

The purposes you collect patient data for, whether each is active, and per-patient consent with a full trail of changes.

The trail is the point. Consent without a dated record of when it was given is not consent you can demonstrate.

02

Patient Rights

Requests from patients to see, correct, export or erase their data — logged, tracked, completed or rejected with a reason.

Erase is here. It is irreversible and it is now admin-only.

03

Breach Register

Recording a data breach, reporting it to the board, and notifying affected patients.

Hopefully never used. Its existence is part of what compliance looks like.

04

Data Retention

What is held, and for how long.

Read-only. There is no execute or purge action — it reports, it does not delete.

THE CORRECTION MADE ON 4 SEPTEMBER, AND WHY IT MATTERED

All four of those screens previously sat behind nothing more than being logged in. There was no permission check on any of them — including the action that **permanently erases a patient's data**. The sidebar hid the links from most roles; nothing actually protected them.

They are now admin-only, enforced. This was the single most serious finding of the whole V1 exercise and it is closed.

Do not teach these screens to reception

The four compliance screens are yours. They are not part of the 14 September staff training, they are not in the Marathi guide, and no member of your team needs to open one. If a patient makes a data request, the correct front-desk action is to bring it to you — not to open Patient Rights.

What it can't do yet

1 · TWO CONSENT SAFETY SWITCHES ARE OFF

There is a setting that would make consent an absolute requirement — never overridable — and another that would make the consent guard refuse rather than allow when it cannot reach an answer. Both are currently off, meaning the guard *fails open*. That is the pragmatic setting for a system going live, and it is a deliberate V1.1 conversation, not an oversight.

2 · THE FOUR COMPLIANCE LINKS SHOW FOR ANYONE WITH SETTINGS VIEW

The routes are admin-only so the direction is safe — a non-admin sees a link and is refused. If you ever grant a manager Settings view, four links will appear that they cannot use.

Status

Corrected in V1. Compliance routes locked to admin on 4 September. Settings itself is unchanged.

A Appendix A • Where Every Number Comes From

One table per screen. If a figure in Dentfluence ever surprises you, the answer is on this page — and in almost every case the answer is that two screens are using different periods or different sides of the invoice.

REFERENCE

ACCURATE AS AT 14 SEPTEMBER 2026

THREE WORDS THAT RESOLVE MOST DISAGREEMENTS

Billed — raised on an invoice. **Collected** — money actually received. **Lifetime** — ignores whatever date filter is on the screen.

A figure marked *Billed · lifetime* and a figure marked *Collected · this month* are not supposed to agree, and the fact that they sit next to each other on the same screen does not make it a fault.

Daily Huddle — the board

FIGURE	WHAT IT COUNTS	BASIS
Collections (Today)	The sum of the <i>amount to collect</i> that a human typed against each of today's scheduled appointments. Walk-ins excluded.	Projected · typed by hand
"x of y appointments logged"	How many of today's appointments actually have that figure filled in. If this is low, the figure above it is meaningless.	Fill rate
Today's Appointments	Appointment rows for today.	Count · today
Today's Actions / Today's Calls	Items on the relationship worklist, with a high-priority and a pending count.	Count · today
Pending Tasks	Open tasks across the team.	Count · open

Daily Huddle — the report tabs

FIGURE	WHAT IT COUNTS	BASIS
Total Collected · Net · Average per day	Corrected in V1 to read the same payment records as the Finance dashboard. Before this release it read a separate ledger that ordinary invoice payments no longer wrote into, and therefore under-counted.	Collected · window
Outstanding balance	Everything unpaid, right now.	Billed · lifetime
Appointments booked / completed / cancelled / no-show	Counts for the window, with rates. Sound — these were never in question.	Count · window
New patients & sources	Registrations in the window, referrals among them, distinct channels.	Count · window
Lab & tasks	Cases sent, received, overdue now, remakes; tasks done, pending, overdue.	Count · window
Protocol compliance	Per person: assigned, done, missed, percentage.	Count · window

Appointments

FIGURE	WHAT IT COUNTS	BASIS
"N patients"	Appointment rows today — not distinct people.	Count · today
The six status chips	Today's rows in that status. <i>They do not sum to the total</i> — Checked Out has no chip.	Count · today
Walk-In chip	Today's walk-ins in any status. Overlaps every other chip.	Count · today
N in queue	Counted in the browser from the visible list.	Count · today
Chair utilisation	Calculated on every page load and shown on no linked page. Assumes 14 hours of capacity a day unless told otherwise.	Not displayed

Patient profile

FIGURE	WHAT IT COUNTS	BASIS
Total Billed	Every invoice total for this patient.	Billed · lifetime
Total Collected · % collected	Paid amount across those invoices.	Collected · lifetime
Outstanding	Balance still due across those invoices.	Billed · lifetime
Advance	Money paid and not yet applied. A liability, not revenue.	Wallet
Recall Status	Nothing. Never calculated, identical for every patient. Ignore it.	Not calculated
Recall on the Consultation tab	A separate calculation from days since last visit, plus six months. Disagrees with the header on the same screen.	Derived in the page

Treatment plans

FIGURE	WHAT IT COUNTS	BASIS
Estimated Investment (saved plan)	Sum of item totals. Each item is fee × units, less item discount, plus GST.	Quoted
Running total while typing	Fee × units only — <i>no discount, no GST</i> . Does not match what will be saved.	Incomplete
Progress chip	Derived from clinical work actually recorded against the plan's items. Ceiling is "all work recorded", never "completed".	Derived

Accounts & Finance — dashboard

FIGURE	WHAT IT COUNTS	BASIS	FOLLOWS THE FILTER?
Collection	Payments received in the period.	Collected	Yes

FIGURE	WHAT IT COUNTS	BASIS	FOLLOWS THE FILTER?
Expenses	Expenses dated in the period, <i>including unpaid bills</i> .	Recorded	Yes
Net Profit	Collection minus Expenses. This is the one profit definition.	Mixed	Yes
Outstanding	Unpaid across every invoice ever.	Billed	No · lifetime
Today's Collection	Payments received today.	Collected	No · today
Avg Daily Collection	Period collection ÷ days elapsed.	Collected	Yes
Projected Period Total	Daily average × full period. A straight line.	Collected	Yes
Collection by mode	Period payments by cash / card / UPI / transfer.	Collected	Yes
Top expenses	Five largest expense categories in the period.	Recorded	Yes
Revenue vs expenses chart	Trailing six calendar months.	Mixed	No · fixed 6 months
Recent transactions	The last ten payments, whenever.	Collected	No · latest 10
Tax estimator	This calendar month's collection × 12, halved for 44ADA, run through the slabs. Quarterly figure is annual ÷ 4.	Indicative	No · this month
Cash In Hand	Removed in V1. Was every cash payment ever, minus every cash expense ever. Never reset.	—	—
Bank Balance	Removed in V1. Was the number zero.	—	—

Accounts & Finance — Income page

FIGURE	WHAT IT COUNTS	BASIS
Today · This Week · This Month	Payments in those fixed windows. <i>They ignore the date range you set for the tables below.</i>	Collected · fixed
Outstanding	Unpaid across all invoices.	Billed · lifetime
Invoices tab	Filtered by invoice date.	Billed · range
Receipts tab	Filtered by payment date.	Collected · range

Revenue by treatment category

FIGURE	WHAT IT COUNTS	BASIS
Billed	Invoice lines in that category, raised in the period.	Billed · invoice date
Collected	Payment allocated against those lines.	Collected · payment date
Outstanding	Billed less collected, <i>for that period only</i> — not the patient's whole balance.	Derived · period

Lab

FIGURE	WHAT IT COUNTS	BASIS
This Month — Estimated / Actual	Cases <i>created</i> this calendar month. A case spanning a month end sits in the month it started.	Cost · created month
Unbilled	Cases with an actual cost, not yet billed by the lab.	Cost · lifetime
Billed	Lab invoices you have not yet paid.	Cost · lifetime
Tab counts	Cases in each state. Clinic-wide, not branch-scoped.	Count · lifetime

Inventory

FIGURE	WHAT IT COUNTS	BASIS
Inventory Value	Available quantity × average purchase price, across active items.	Value · now
Low / Critical / Out of Stock	Against each item's minimum quantity. <i>Items with no minimum set are excluded entirely.</i>	Count · now
Expiring Soon	Distinct items with stock expiring within 90 days.	Count · 90 days
Days Until Stock-out	Derived from the last 30 days' consumption.	Derived
Dead Stock Value	Value of items with no movement in 90 days.	Value · 90 days
Monthly Wastage	Expired plus damaged, this calendar month.	Value · month
Health Score	100 minus capped deductions for out, low, expiring and dead stock. Optimistic — items with no minimum count in the denominator and never deduct.	Index
Value Trend chart	<i>Not inventory value</i> — purchases per week for eight weeks.	Spend · weekly

Tasks · HR · Prescriptions

FIGURE	WHAT IT COUNTS	BASIS
Tasks: Overdue / Due Today / Upcoming	Open tasks by due date, for the branch.	Count · now
Tasks: Completed	Captioned "this period" and has no period. Every completed task ever.	Count · lifetime
HR: Present	Present, late and half-day together.	Count · that day
HR: Not Marked	Total staff minus rows recorded. Can go negative.	Count · that day

FIGURE	WHAT IT COUNTS	BASIS
Prescriptions: Total	Includes deleted prescriptions — the other three cards do not.	Count · lifetime
Prescriptions: Draft	Permanently zero. There is no draft step.	Always 0

Reports & Analytics

FIGURE	WHAT IT COUNTS	BASIS
Reports → Collected	Payments in the selected window. Same definition as the Finance dashboard.	Collected · window
Reports → Outstanding	Unpaid across all invoices.	Billed · lifetime
Reports → Collection rate	A period figure divided by a lifetime one. Do not use it.	Invalid
Analytics → expenses	Only expenses you have <i>paid</i> — unlike the Finance dashboard, which includes unpaid bills. The two will differ.	Paid only
Analytics → Audit & History summary	Sums only the rows shown — the latest 50 payments, 50 vouchers, 20 reconciliations. Not period totals.	Sample
PRE → relationship score	Zero, for everyone, always. Never calculated.	Not calculated

THE SEVEN FIGURES IN THIS PRODUCT THAT SHOULD NOT BE READ AT ALL

Collected in one place so you can say them out loud in the training room:

1. Patient profile → Recall Status · 2. PRE → Relationship Score and the score distribution chart · 3. Prescriptions → Draft · 4. Reports → Collection rate · 5. Analytics → Audit & History summary totals · 6. Tasks → Completed "this period" · 7. Any money figure on a treatment visit card.

None of them is being fixed before 14 September. All seven are V1.1 candidates, and the right V1.1 decision for most of them is to hide the figure rather than to compute it.

B Appendix B · Built, and Switched Off

What exists in the system and is deliberately not part of V1. Written down so that nobody switches one on by accident, and so that V1.1 starts from a list rather than from memory.

NOT IN V1

REFERENCE

The mobile app

NOT IN V1. NOT TO BE DISTRIBUTED. NOT TO BE MENTIONED.

An Android app exists. Every file in it was last changed on **15 July 2026** and the last build is from the same day. In the seven weeks since, the web system has moved on and the interfaces the app talks to have changed. It has fifty-three screens and *no role restrictions of any kind* — every screen is visible to every user who logs in.

The July build goes to nobody. Mobile is V1.1, dated 30 September, and it has been redefined as a data-entry layer for seven specific jobs rather than a copy of the web system. The website and the Instagram page must not promise an app.

Case Acceptance — the patient microsite

SWITCHED OFF IN PRODUCTION ON 4 SEPTEMBER. IT HAD BEEN ON SINCE JULY.

This feature presents a treatment plan to the patient on their own phone, through a link, so they can accept or decline it themselves. The pages it serves sit *outside the login* — anyone holding a valid link could open them, by design, because the patient is not a user of your system.

You parked this feature on 28 August, and every document since assumed it was off. Nobody had checked. It was on, and had been since July. It was switched off and verified off on the evening of 4 September. One patient record was affected.

It is a genuinely good feature and it should come back in V1.1 — deliberately, with a decision about link expiry, rather than because a setting was left on.

Automated WhatsApp

Covered in Chapter 8. In short: no credentials exist on the production server, Meta business verification has not started, and until it completes no template message can leave the system. Three templates have a working send path waiting behind that wall; four have no send path at all. The one thing that works today is the click-to-chat button that opens WhatsApp on a staff member's own phone.

The Communication module

A full communication system exists in the codebase — threads, a timeline, an inbox. It is unregistered and unreachable, and it was switched off in July because the timeline was displaying fabricated patient conversations. It stays off. This is the precedent behind the Marketing column removal in Chapter 1: a screen that invents content is worse than a screen that is missing.

The Marketing module

Marketing exists as a module with its own permission, and the Blog Marketing Hub within it went live in August and is deliberately switched on. What is *not* in V1 is any connection between Marketing and the rest of the product — in particular, the Huddle's Marketing column, which showed four invented posts and has been

removed. The slot on the board is reserved for V1.1, reading genuinely scheduled content.

Feature switches

The system has twenty-eight switches for capability that is built and not turned on — a rules engine, an automation engine, a workflow engine, a unified notification store, search indexing, and integrations with Google, Meta, payments and ABDM. **Exactly one is on by default**, the one that links a new patient into the relationship spine. Two more are on deliberately in production: the Blog Hub, and — until 4 September — Case Acceptance.

THE RULE THAT FOLLOWS FROM THIS

Twenty-seven switched-off capabilities is not technical debt; it is a product that was built ahead of its clinic. But it does mean the honest answer to "can Dentfluence do X?" is often "the code exists, it has never run against your data, and switching it on is a decision with consequences".

Nothing gets switched on without a row in the launch tracker. The 4 September finding — a feature everyone believed was off having been live for six weeks — is the whole argument for that rule.

Built and never wired

WHAT	STATE
Team Accountability screen	Fully designed, never connected. The link to it is being removed in V1.
Huddle notes and comments	Database tables and save routes exist; there is no box on the board to write in.
Chair utilisation	Calculated on every page load, displayed only on a page with no link.
Five Finance pages	Reports, GST, Cashbook, Payroll and the Voucher register — working, unlinked.
Four analytics projections	Rebuilt every fifteen minutes and read by nothing.

WHAT	STATE
Automatic plan suggestion	Exists, unreachable, and should stay that way — it is keyword matching, not intelligence.
Smart Presentation module	Permissioned, routed, and removed from the sidebar in July.
The AI copilot	Off in production, as intended.

What V1 actually is

STATE IT THIS WAY TO ANYONE WHO ASKS

Dentfluence V1 is the web system Tulip already runs every day, with every number on the money and huddle screens verified true, every fake and dead surface removed, admin routes locked down, role permissions enforced on writes with an activity log, and a written Owner's handbook and Marathi staff guide.

One clinic. Web only. No new modules.

And three things it is explicitly not: not the Android app, not automated WhatsApp, and not a full KPI dashboard.

Dentfluence Owner's Handbook · Complete edition · 16 chapters and 2 appendices · Chapter 1 written 27 August 2026 and revised 4 September; chapters 2–16 and the appendices written 4 September 2026, against the **post-correction** state of the system as it will stand on 14 September — not against the screens as they were on the day of writing.

Every claim in this book was read out of the running Laravel codebase, not from a specification. Where a figure is wrong, this book says it is wrong. Where a feature does not exist, this book says it does not exist. Nothing here promises anything that has not been verified.

Companion volumes: the **Staff Guide** (Marathi, for the floor), the **V1 Decision Log**, and the **Go-Live Rollout Plan** for 14 September.